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BMJ Journals style guide and references

Abbreviations

Abbreviations should always be written out in full the first time they appear. If defined in an abstract, they must be defined again on first use in the main text of a paper; similarly, they should be defined in boxes, figures and table captions even if they have already been defined in the main text.

Examples:

'Major depressive disorder (MDD) is associated with significant morbidity and mortality.'

'The aim of this study was to investigate whether axonal excitability indices are associated with survival in patients with amyotrophic lateral sclerosis (ALS).'

'The present scale, the Upper Limb Assessment in Daily Living (ULADL) Scale, was developed...'

Abbreviations in manuscript titles

Leave as supplied by the author and spell out the abbreviation in the text instead. However, if the abbreviation makes the title ambiguous then the abbreviation should be spelt out.

Note: There is no need to add an abbreviation after words if they are only used once in the manuscript. For example, if the National Health Service is used only once in a paper it doesn't need the abbreviation (NHS) after the words.

Genus names

There should always be a full point after contracted genus name, eg, *E. coli*, followed by the species name. Genus should always be spelt out in full at first mention in an article (follow rules for other abbreviations). All should be italics: *Helicobacter pylori* (second mention: *H. pylori*).

alpha, beta, gamma	Leave as author provides (but ensure there is consistency throughout article)
Abbreviation at beginning of sentence	When a sentence begins with an abbreviation the next word does not begin with a capital letter. Example: 'HIV antigen ...' If a quote uses an abbreviation spell it out [in square brackets] afterwards.
US states	Spell out US states in full (except when there is a zip code in an address). DC is the exception (Washington, DC).
Abbreviations that are treated as words	In general, if abbreviations are used and they 'sound like words', do not precede with 'the.' Example: 'NICE', not 'the NICE'; 'BJSM', not 'the BJSM.'
eg and ie	for example, eg, e.g. <i>Action:</i> eg (for example) and ie (that is) should be spelled out in full if used in main text, unless within parentheses. When used, please add a comma. Example: "When cats are fed (eg, morning and evening) ..." etc, etc. 'etc' can be used in the main text, tables, figures and boxes (as space saving). Do not spell out in full (etcetera). No full point after 'etc'

BMJ Journals style guide and references

Action: ie (that is) should be spelled out in full if used in main text, unless within parentheses. When used, please add a comma, no full points between the letters ie. that is, ie, i.e.,

tt

Abbreviations in boxes, figures, tables, subheadings

Abbreviations must be redefined within each table, box and/or figure. In figure legends, abbreviations and their definitions should be listed in alphabetical order. Abbreviations can be spelt out in full and abbreviated in parentheses on first use (space permitting), otherwise, define in footnotes:

Example of a footnote definition: ALS, amyotrophic lateral sclerosis; HCV, hepatitis C virus.

~~Leave abbreviation (and no other text), please spell it out in full (even if it has already been abbreviated beforehand):~~

~~Examples:~~

~~Hepatitis C virus (should be spelled in full).~~

~~HCV and children (this is OK).~~

Abbreviations that do not need defining

No need to spell out	
ACE - angiotensin converting enzyme	HIV - human immunodeficiency virus
ADP - adenosine diphosphate	H&E - haematoxylin and eosin
ADP - adenosine diphosphate	HR - hazard ratio
AIDS - acquired immunodeficiency syndrome	ID, identification number, etc
AMP - adenosine monophosphate	IQ - intelligence quotient
ATP - adenosine triphosphate	IQR - interquartile range
BB, MB, MM - There are several types of creatine kinase: BB (brain type), MB (heart type), MM (muscle type); no need to spell out these three abbreviations.	MRI - magnetic resonance imaging
BCG - Bacillus Calmette-Guérin	OR - odds ratio
C4 - C4, etc (complement)	PCR - polymerase chain reaction
CI - confidence interval	RNA - ribonucleic acid
CT - computed tomography	SARS-CoV-2 - does not require expansion.
COVID-19 (Coronavirus / COVID-19 / Covid-19 / covid-19 / SARS-CoV-2, Use all caps for the term for ALL articles across ALL journals). Note, "long covid" should be all lower caps.	SD, sd, S.D., s.d. - standard deviation, change all to SD
chatGPT	SE, se, S.E., s.e. standard error, change all to SE
	SEM, sem, S.E.M., s.e.m. standard error of the mean, change all to SEM
	SMS - short messaging system

BMJ Journals style guide and references

DNA - deoxyribonucleic acid	SPSS - Statistical Product and Service Solutions
ECG - electrocardiogram	UNICEF, UNESCO - (All caps)
EDTA - ethylene diamine tetraacetic acid	WHO - World Health Organization
ELISA - enzyme-linked immunosorbent assay	

Acronyms

Important organisation acronyms:

EULAR

European Alliance of Associations for Rheumatology

ACR

American College of Rheumatology

ACR should be spelled out in full in the title and at first mention in the text. Thereafter, the ACR acronym should be used.

=====

~~Description of the acronyms and full names of the reporting guidelines shown in the EQUATOR website as 'Reporting Guidelines for main study types':~~

~~Copy editors should choose the definition from this list and ask authors to confirm the definition is correct rather than asking authors to define the acronym.~~

CONSORT

Consolidated Standards of Reporting Trials

STROBE

Strengthening the Reporting of Observational Studies in Epidemiology

PRISMA

Preferred Reporting Items for Systematic Reviews and Meta-Analyses

SRQR

Standards for Reporting Qualitative Research

COREQ

Consolidated criteria for Reporting Qualitative research

TRIPOD

Transparent Reporting of a multivariable prediction model for Individual Prognosis or Diagnosis

SQUIRE

Standards for Quality Improvement Reporting Excellence

EQUATOR

Enhancing the QUALity and Transparency Of Health Research

BMJ Journals style guide and references

SPIRIT

Standard Protocol Items: Recommendations for Interventional Trials

PRISMA-P

Preferred Reporting Items for Systematic Review and Meta-Analysis Protocols

CARE

Case Report

AGREE

Appraisal of Guidelines, Research and Evaluation

RIGHT

Reporting Tool for Practice Guidelines in Health Care

CHEERS

Consolidated Health Economic Evaluation Reporting Standards

Abstracts

(note, this refers to abstracts at the start of articles – NOT meeting abstracts)

References are not allowed in abstracts

For information on whether an abstract should be structured or unstructured refer to the Master How to guide.

Abstract headings

Do not include colons after the subheadings (eg, 'Background:' should be 'Background', 'Methods:' should be 'Methods', etc.

Trial registration and PROSPERO registration numbers

If Trial registration/PROSPERO registration numbers are provided in the manuscript, typeset at the end of the Abstract as “Trial registration number” or “PROSPERO registration number” (and ALSO retain it in the manuscript as supplied).

Full instructions are provided [here](#).

FYI: PROSPERO is a database of systematic review protocols.

Personal pronouns in abstracts

Authors are permitted to use personal pronouns in abstracts; for example, 'Our study investigated' or 'We examined 50 cases...' —these are acceptable.

BMJ Journals style guide and references

Keywords

Keywords can be left as supplied by the author. All journals request authors to select their keywords in Sch1 from the MeSH predefined list, therefore we do not require copy editors to check them for:

- spelling mistakes;
- US/UK English;
- standardisation

Capital letters

Committees within organisations take lowercase initial letters; the organisations themselves (when given their proper name) take capitals. Thus: the education committee of the Royal College of Nursing, the public health department of Manchester Health Authority, the social services committee of Camden Council, the scientific and technical branch of the DH.

Use capitals for all regional and district health medical authorities, family practitioner committees and local authorities when given their full proper names. When mentioned in generality change to lowercase when only part of the name is used; for example, the Royal College of Paediatrics and Child Health and the Royal College of Psychiatrists - use 'the colleges' (the word colleges is lower case because it is referring to multiples, not just one specific college). However, when used in specifics the cap remains; for example, the Royal College of Paediatrics and Child Health - use 'the College' (this is referring to just this particular college and is an abbreviation of the full title of the College, and so is capitalised).

Upper case

Databases, registers, programmes, projects, initiatives, studies, questionnaires, surveys, scales, instruments, scores and indexes.

Examples: Trent Regional NHS Hospital Admissions Database; Cochrane Controlled Trials Register; General Health Questionnaire; Glasgow Coma Scale.

Names remain initial capitals: Examples: President Obama; the Duke of Argyll; God; the Queen.

Brand names take an initial capital, generic names do not: Example: Panadol (brand); paracetamol (generic).

English, French, etc, are capitalised (because they are names of languages); All specific wars have capitals; for example, World War I.

Lower case

For sentences that start with a number or a lower case letter the first letter of the next word should be capitalised:

Examples: p Value; 3-Hydroxymethane

(Exception: in the abstract, sentences can start with a numeral (there is no need to rearrange the sentence or spell out in full). The first letter of the next word does not need to begin with a capital letter, eg: 249 patients were...).

All government positions are lower case:

Examples: prime minister; secretary of state for health; the cabinet.

act (unless named); bill (unless named); government; green paper; member of parliament; opposition; parliament.

BMJ Journals style guide and references

Computing terms should be lowercase: internet, email, web page, etc.

anglophile, francophile, gallic.

~~Open access~~

~~We always refer to open access in lower case (vs Open Access) in all of our articles.~~

~~All occurrences of 'Open Access' should be changed to sentence case. This will mean that if it appears in the section headings it should be 'Open access'. In the open access statements at the end of an article it should be:~~

~~Open access This is an open access article distributed in accordance with the...~~

Compound words

Re-phrase '2-4-day-old babies' to 'babies aged 2–4 days' (include en rule for range).

'2- to 4-day-old babies' is not allowed.

In fractions, three-quarters, two-thirds, etc. Note: 'a quarter' is OK.

Computer language, internet, urls

Program - refers to computer program (otherwise spell as programme).

Program formats

SPSS, PowerPoint, Word, etc.

Program versions

V.1 eg Microsoft Windows V.XP.

TikTok, Instagram, Snapchat, WhatsApp, Windows 11

Only need to author query if a version is not provided for SPSS, STAT, SAS or Windows (no need to query for other programs if version is not provided).

No need to include the place of manufacture.

URLs

URLs start with http://, unless ftp:// or other such designations. Add URL prefix if its not provided.

Avoid ending a sentence with a URL (either rearrange the sentence so that the URL is not at the end of the sentence or put the URL in parentheses).

URLs in references - no full stop at the end of a URL.

When a URL breaks across lines do not insert a hyphen as this makes the URL incorrect.

Websites of recognised organisations may be cited in the text without an access date; otherwise the URL should be in the reference list with 'author', 'title' and 'access date' (see references guide).

Access dates - at the end of URLs access dates should be in the following format:

(accessed 12 Feb 2012).

No need to author query if it hasn't been supplied by the author.

BMJ Journals style guide and references

Copy editing – specific journal requirements

BMJ Military

The following words will always be in capital letters as they are mission codes:

Op BANNER	Op CORPORATE
Op GABRIEL	Op GRANBY
Op GRAPPLE/RESOLUTE	Op HERRICK
	Op TELIC

Use capital for 'operation' and 'exercise' only when it directly precedes an actual operation name, ie 'Exercise SS3', 'Operation HERRICK'.

Abbreviations that don't need to be spelt out in full (same as EMJ):

NHS

AXR = abdominal x-ray

ALS = advanced life support

ATLS = advanced trauma life support

CT = computerised tomogram or tomography

ECG = electrocardiogram

HR = heart rate

RR = respiratory rate

USS = ultrasound scan

ABG = arterial blood gas

APLS = advanced paediatric life support

BP = blood pressure

CXR = chest x-ray

GCS = glasgow coma scale or score

MRI = magnetic resonance imaging

SXR = skull x-ray

BMJ Case Reports

Cases uses the ADC style for differentiating ages of children so that the actual age is not stated. Exact patient ages should never be published in any case reports.

Preterm neonatal - The period at birth when a new-born is born before the full gestation period

Term neonatal Birth 27 d

Infancy 28 d–12 mo

Toddler 13 mo–2 y

Early childhood 2–5 y

Middle childhood 6–11 y

Early adolescence 12–18 y

Late adolescence 19–21 y

BMJ Global Health

On all other journals we ask Exeter to omit the opening 'article' words (eg, 'The', 'A', 'An' etc) in titles; however, please DO NOT omit these words in this journal (specific Editor request).

British English.

Please break up the bullet points (key questions) into two, so that it becomes one sentence for each bullet point, i.e. ensure that there is only one sentence in each bullet point in the summary box.

BMJ Medicine

Very complex. For now, see separate [How To do](#) document shared with tBMJ team.

BMJ Open Diabetes Research and Care (DRC)

US English (if the author uses oxford comma, leave it in. If they don't use the oxford comma, leave the text as it is).

Do not change 'subjects', 'participants', 'volunteers', 'people' and 'patients' Please leave as supplied by the author.

BMJ Journals style guide and references

BMJ Open Gastroenterology (BMJ OG)

British English (if the author uses oxford comma, leave it in. If they don't use the oxford comma, leave the text as it is).

BMJ Open Sports and Exercise Medicine (BMJ SEM)

British English

Abbreviations that don't need to be spelled out in full: ACL, FIFA, NBA, NFL, NHL, WADA, IOC, UEFA

BMJ Surgery, Interventions, & Health Technologies (BMJ SIT)

US English (please retain Oxford commas. If the Oxford comma is not used, leave the text as it is).

Drugs and Therapeutics Bulletin (DTB)

British English

~~Emergency Medicine Journal (EMJ)~~

~~Abbreviations that don't need to be defined within the text. If they appear in the title they will need to be expanded.~~

~~NHS = National Health Service~~

~~GCS = glasgow coma scale or score~~

~~ALS = advanced life support~~

~~APLS = advanced paediatric life support~~

~~ATLS = advanced trauma life support~~

~~ECG = electrocardiogram~~

~~BP = blood pressure~~

~~HR = heart rate~~

~~RR = respiratory rate~~

~~ABG = arterial blood gas~~

~~CXR = chest x ray~~

~~CT = computerised tomogram or tomography~~

~~MRI = magnetic resonance imaging~~

~~USS = ultrasound scan~~

~~AXR = abdominal x ray~~

~~SXR = skull x ray~~

~~A&E (UK abbreviation) = Accident and Emergency~~

~~ED = Emergency Department~~

Heart

Paper titles: No abbreviations to appear in paper titles, they should be defined.

Journal of Medical Genetics (JMG)

Gene names must be in italic. Protein names in roman.

Abbreviations that don't need to be spelled in full:

A, C, G, T, U (only in the context of a DNA or RNA sequence)

Adenine, Cytidine, Guanine, Thymine Uracil

ACE inhibitors angiotensin converting enzyme

ADP adenosine diphosphate

AIDS acquired immunodeficiency syndrome

BMJ Journals style guide and references

AMP	adenosine monophosphate
ATP	adenosine triphosphate
BAC	Bacterial artificial chromosome
BCG	Bacillus Calmette-Guérin
Bp	Base Pair
C4	C4, etc (complement)
cDNA	Complementary DNA
CI	confidence interval
CNV	copy-number variant
DNA	deoxyribonucleic acid
ECG	electrocardiograms
EDTA	ethylene diamine tetraacetic acid
EEG	electroencephalogram
ELISA	enzyme-linked immunosorbent assay
EST	expressed sequence tag
FISH	Fluorescent in situ hybridisation
H&E	haematoxylin and eosin)
HIV	human immunodeficiency virus)
HPLC	high-pressure liquid chromatography (P may also stand for “performance” but it means the same method, no difference).
IQ	IQ (intelligence quotient)
IQR	interquartile range
kb, Mb, Gb	kilo- mega- giga- base
LD	linkage disequilibrium
M, cM	Morgan, centiMorgan
miRNA, microRNA	
MRI	magnetic resonance imaging
mRNA	messenger RNA
OMIM	on-line Mendelian inheritance in man
OR	odds ratio
PCR	polymerase chain reaction
Q-PCR or QPCR	quantitative PCR
RNA	RNA (ribonucleic acid)
rRNA	ribosomal RNA
RT	reverse transcriptase
RT-PCR	reverse-transcription-PCR
SD	standard deviation
SE	standard error)
SEM	standard error of the mean)
shRNA	short hairpin RNA
SNP	single-nucleotide polymorphism
SSCP	single-strand conformation polymorphism
sSNP, nsSNP	synonymous, non-synonymous SNP
SSR	short sequence repeat
tRNA	transfer RNA
Unicef, Unesco	Use as words (ie not all caps)
UPD	uniparental disomy
WHO	World Health Organization)
YAC	yeast artificial chromosome

BMJ Journals style guide and references

Journal of NeuroInterventional Surgery (JNIS)

US spelling. Oxford commas (cats, rats, and bats) and US dates (November 28, 2009) [except for the Received and Accepted dates]. If the author uses the Oxford comma, leave it in. If they don't use the oxford comma, leave the text as it is. P values are always written as P values and not with a lowercase p.

UK based copy editors only.

Abbreviations that don't need to be spelled out in full:

DSA (Digital Subtraction Angiogram),

IA (Intra-arterial),

IV (Intravenous).

Lupus

British English

Abbreviations that do not need expansion: SLE, ANA.

Medical Humanities (MH)

Page numbers for Vancouver referencing

These should be included after the citation number, in parentheses, without a space in between, in superscript

Example of one page listed:

- Only one page will be listed after this sentence.^{1(p101)}

Example of a range of pages or multiple pages listed:

- A range of pages will be mentioned at the end of this sentence.^{2(pp40-5)}
- Finally, multiple pages will be listed here.^{3(pp3,5)}

Practical Neurology (PN)

Abbreviations that don't need to be spelled out in full:

CT scan is fine (does not need to be 'computed tomogram')

MRI to be changed to MR (MRI as a standalone acronym is fine; does not need to be 'magnetic resonance imaging')

PCR is fine (does not need to be 'polymerase chain reaction')

DNA is fine (does not need to be 'deoxyribonucleic acid')

Note: "neurone" should not be spelled "neuron". "neurone" is the correct spelling for this journal.

All articles in Practical Neurology should have a named referee if externally peer reviewed.

Provenance and peer review statement added to the export note from SC1 - these should be manually added to the paper (eg, Provenance and peer review. Not commissioned. Externally peer reviewed by Martin Zeidler, Edinburgh, UK, and Simon Mead, London, UK)

Trauma Surgery & Acute Care Open (TSACO)

US English

Units of measure should be expressed in the metric system.

SI units should be used throughout, except for blood pressure values, which should be reported in mm Hg.

Please see the TSACO [Style guide](#) for exceptions to BMJ Style.

References should be *et al* only after 10 authors.

BMJ Journals style guide and references

Corrections - see Combined corrections doc

Generally, corrections follow the format:

The heading 'Correction'.

The doi of the paper that is being corrected followed by 'Corr1'

First three author names followed by '*et al.*'

Paper title.

Issue details (when the paper appeared in print).

Correction details.

Standard correction example

Correction

doi:10.1136/jnnp-2011-300148corr1

Andersson M, Alvarez-Cermeño J, Bernardi G, *et al.* Cerebrospinal fluid in the diagnosis of multiple sclerosis: a consensus report (J Neurol Neurosurg Psychiatry 2011;5:897–902). The last five authors were missing from the online version of this paper. This omission has been rectified and the authors are now all credited.

Abstract book correction example

doi:10.1136/adc.2011.212563.2corr1

McAree T, Manickavasagar T, Sivalokanathan S, *et al.* High levels of vitamin D deficiency in pregnancy – a failure of public health policy? Arch Dis Child 2011;96:A1. doi:10.1136/adc.2011.212563.2. The order of the authors was published incorrectly in this abstract. The correct order is: etc.

Post PAP corrections

When PEs deem it appropriate to correct an article that has already been published Online First, as well as getting the article amended, the typesetters should be instructed to add a footnote to the new version at the end of the article (before the 'Competing interests' etc; same font & style).

This footnote always starts with: 'This article has been corrected since it was published Online First.'

Examples:

Correction notice This article has been corrected since it was published Online First. Figure 2 has been corrected.

Correction notice This article has been corrected since it was published Online First. The authors' competing interests have been added.

Sometimes more description is needed:

Example:

Correction notice This article has been corrected since it was published Online First. In the Methods, paragraph 2, the number of patients was 200; not 20.

The correction notice will remain in version and go into print issue with it too.

Countries and places

Leave as author supplies in author affiliation and corresponding author addresses.

United States of America

When referring to the United States of America, use US adjectively and USA when referring to the country.

BMJ Journals style guide and references

American spelling

Leave American spellings in official names and departments, as supplied by the author (exception is JAMIA, which takes American spelling).

Example: Department of Pediatrics, Centers for Disease Control and Prevention, etc.

Great Britain, UK, etc

Britain is a non-specific term; be precise if possible.

Great Britain comprises England, Scotland and Wales.

Dates and time

Date format

All journals using British English should follow the UK date format, ie, day/month/year.

US journals which use American English should use US date format, ie, month/day/year

Be careful when converting 'American' dates for a British English journal: eg, 9/11 is 11 September (not 9 November).

Ordinal indicators in dates and numbers

Suffixes -st, -nd, -rd, -th in dates should be removed, ie, 2 November 2021 not 2nd November 2021.

When ordinal numbers are used to indicate the position of something in a list, such as 1st, 2nd, 3rd, 4th, 5th etc please retain the suffixes. Most ordinal numbers end in "th" except for: one ⇒ first (1st) two ⇒ second (2nd).

Dates in references

Abbreviate months in references (accessed dates); eg, 'Jan', 'Feb', 'Mar', etc.

years

Use 'from 1975 to 1981' (not: 'from 1975–1981').

Do not contract years (not: '1975-81').

days

Follow Chambers for use of capitals (New Year's Day, New Year's Eve, Christmas Eve, Christmas Day, Mother's Day, Mothering Sunday, St Valentine's Day).

weeks/months

In full; do not abbreviate.

hours

Spell out in full, do not abbreviate to 'h'.

minute/minutes

Abbreviate to 'min'. When it is used adjectivally then it should be spelled out in full and hyphenated, eg, 2-minute performance

second/seconds

Abbreviate to 's'.

twice daily, thrice hourly, etc

BMJ Journals style guide and references

Change to 'two times per day'; 'three times an hour', etc.

time of day

Time of day: use 24-hour clock 09:00, 18:30, 21:45

Patient history

Patients present with a XX-day/XX-month/XX-year history.

For example: 'The patient presented to the emergency department with a 3-month history of severe coughing.'

Do not use the word 'back' use the word 'ago'; for example, 'The patient was apparently well until 5 months ago'. The word 'back' is slang and should not be used.

BMJ Journals style guide and references

Drugs/medication

Correct	Incorrect/further clarification
complementary medicine	alternative medicine
contraceptive pill/oral contraceptives	oral contraceptive pill
drug misuser	drug abuser
epinephrine	adrenaline
estradiol is the drug	oestradiol is the naturally occurring substance (ie, in the body).
morphine	morphia
norepinephrine	noradrenaline
'was given aspirin' or 'started taking aspirin'	commenced on aspirin

BMJ Journals style guide and references

Endash and emdash and hyphen rules

– en rule

We use this to signify ‘and’ or a ‘range’ between any two items. In regard to author names or between names of any two people, we use hyphens. Compound nouns use hyphens

Example of a range:

“7.35–7.45”

“133–146mmol/L”

Example of ‘and’: “Heart rate was 60–80 beats per minute.”

However, use ‘to’ instead of an en rule when both values are negative? Otherwise it looks a bit confusing:

Example ‘–1.6—2.3’ would look better as –1.6 to –2.3

Example compound nouns: “A term 2-week-old male infant presented to the emergency department with a 24-hour history”

Example of hyphenated name: Malcolm Brown-Smythe

— em dash

We use this for parenthetical commas or for parentheses.

Example for parentheses: “Ideally, a blood sample is collected around the time of diagnosis—once hyponatraemia and hyperkalaemia have been confirmed—for aldosterone and renin concentrations.”

Example for parenthetical commas: “Rare: Ppseudohypoaldosteronism type 1—this is caused by autosomal dominant or recessive mutations in the genes encoding the mineralocorticoid receptor”

Example for parenthetical commas: In a healthy human, to maintain a steady state almost the entire daily intake of potassium is excreted—approximately 90% via urine...

If a hyphen is used as a parenthetical dash in an article title, it should be changed to an emdash

Examples:

Education & Practice uses the emdash with no spaces either side.

Fifteen-minute consultation: Management of mammalian bites in children—from local wound care to prophylactic antibiotics

Anais Lucile Schneider, Céline Ritter Schenk, Petra Zimmermann

doi: 10.1136/archdischild-2023-326403

Gut uses the emdash with no spaces either side.

ForePass endoscopic bypass device for obesity and insulin resistance—metabolic treatment in a swine model

doi: 10.1136/gutjnl-2023-331335

BMJ Journals style guide and references

Ethics approval statements

For the journals:

BMJ Open (Protocol)
BMJ OG (Protocol) and
BMJ Ophthalmology (Protocol),

Exeter should include the ethics approval statement in the abstract. If it is also provided in the main text, it should be relocated to the end matter (except BMJ Open 'Protocol'), specifically for these article types.

For other journal categories, if the ethics statement is provided in the body text then Exeter should move the statement to the endnotes and remove it from the main text.

For the other journal categories, if the ethics statement appears in both the endmatter and the abstract, the statement should be placed solely in the end matter.

Figures, tables and boxes

Boxes

All boxes should be numbered, ie, Box 1, Box 2, etc (not counting summary points boxes). If there is only one box this should be called Box 1.

Use bullets in boxes for each point/paragraph, even if it contains only one point.

Figures and tables

Spell out 'figure' in full (not 'fig') (when referring to an illustration).

Do not capitalise 'figure' or 'table' within the text except when the figure or table starts a sentence.

Do not capitalise 'table' within text (ie, 'table' and not 'Table').

Major problems should be flagged, examples of these include: low resolution, illegibility and spelling errors.

If bold type is used in data in a table but a footnote is missing explaining the reason for bold type then an AQ should be raised with the author.

All graphic files should be named Figure, ie, even if it's a graph, forest plot, infographic, photograph, illustration then they should be renamed as a figure. For example, graph 1 should be typeset as figure 1.

Figure legends

Rather than '(adapted from [4])', include the author(s) of the reference – '(adapted from Venables et al[4])'.

Multiple figures

Refer to multiple figures as, eg figure 1A–D, figure 1A,B.

Part labels casing in figures and legends

Part labels must be consistent with the corresponding figures.

Part labels should be uniform across all figures within the same article.

If any discrepancies are found between the figure body and the caption casing, please update the caption to align with the figure label casing and ensure consistency throughout the article.

Data alignment in tables

Data in tables should be left aligned

Table caption should be appear in sentence case with no end period

BMJ Journals style guide and references

Example: Table 1 Summary of study characteristics

Table column heads: Sentence case

Examples: Linear model; Study design; Number of women with depression

When tables or figures are taken from a published source, permission should be mentioned.

When tables or figures are adapted from a published source, acknowledge that they have been adapted; give reference number in legend. No need to seek permission.

Tables in an appendix should be numbered separately from those in the paper itself (eg, tables AI, AII, etc).

Appendices should be online only (but if the author has seen and approved in proof, seek author's permission for it to go online only).

Full points should be retained at the end of full sentences, do not delete them.

Table headers

Example: Change 'Number of injuries' to 'Injuries (n)'. It's neater and saves space.

Table cell entries

NA, N.A., N/A, na, n.a., n/a - Leave as author provides (but ensure there is consistency throughout article)

NS, N.S., N/S, ns, n.s., n/s

Action: Leave as author provides (but ensure there is consistency throughout article)

Footnotes to tables and figures

Use commas and semi-colons (rather than em dash or the equal symbol (=) for lists).

List abbreviations in alphabetical order.

Example:

ALS, amyotrophic lateral sclerosis; HCV, hepatitis C virus; MMD, moyamoya disease.

BMJ Journals style guide and references

Footnote symbols

Footnote symbols are full size (not superscript) in the body of tables.

Table footnotes are not followed by a space, eg: *p<0.05.

Order of footnotes

Order of footnote symbols is: *, †, ‡, §, ¶, **, ††, ‡‡, §§, ¶¶, etc.

In figures, footnotes run on.

In tables, new line per footnote for unlinked footnotes and for those denoted with a symbol (unless too little text per footnote and is a waste of space).

Abbreviations on one line, semi-colon separated, and in alphabetical order).

Footnote examples:

Extra information re table that is not linked by a symbol.

*Linked information re footnote with first symbol.

†Linked information re footnote with second symbol.

MD, Maryland; NY, New York; UT, Utah.

(Note: the abbreviations 'eg' and 'ie' can be used in footnotes).

P values

Conventional p value and footnote symbols are: *p<0.05, **p<0.01, ***p<0.001.

If the p value is not one of the above conventional values, use the standard footnote order.

Example: if 'p<0.05' is included as a footnote, any other style footnote must begin with the standard footnote sequence: †, ‡, §, ¶, ††, ‡‡, etc.

Textual footnotes

For textual footnotes use lower case roman numerals (superscript i. ii. iii, etc).

Try to avoid footnotes where possible (exception is JME, which often includes textual footnotes).

BMJ Journals style guide and references

Funding statements

When the Funding comes through as 'No' in the S1 xml output file for the NLM article types:

- Brief report
- Case report
- Editorial (unless it's the Editor's usual 'in brief' type piece)
- Rapid communication
- Research article
- Review article

Please include the statement:

"The authors have not declared a specific grant for this research from any funding agency in the public, commercial or not-for-profit sectors."

For all other NLM article types (where the Funding comes through as 'No') please DO NOT include a Funding statement.

Supplementary files

~~Typesetters should always check that any supplemental files supplied are 1) not a patient consent form or 2) corrupt or 3) does not seem to match with what is referenced within the article.~~

Research checklists

There are no legal/ethical concerns with publishing the research checklists. So please continue to treat these as normal supplementary material.

1. PRISMA checklist
2. CONSORT checklist
3. STROBE checklist

Exeter does not need to raise a query for the author in case no citations to these checklists are provided.

BMJ Journals style guide and references

General

The majority of our journals use UK English and UK spelling.

The following journals are the exceptions and require **US English** to be used

BMJ Open Diabetes Research and Care (DRC)

International Journal of Gynecological Cancer (IJGC)

Journal for ImmunoTherapy of Cancer (JITC)

Journal of NeuroInterventional Surgery (JNIS)

Regional Anesthesia & Pain Medicine (RAPM)

World Journal of Pediatric Surgery (WJPS)

Our preferred dictionary is Chambers 21st Century Dictionary

<http://www.chambers.co.uk/dictionaries.php>

We do not use full points after author names or after Dr, etc, et al; nor do we use full points in 'eg' or 'ie' - these should be replaced with 'for example' and 'that is' respectively, if possible.

BMJ Journals style guide and references

Hyphens and rules

Hyphens are not needed in constructions such as, 'a naturally occurring substance', or (Fowler) 'a prettily furnished room'. Nor are they required after words such as 'well' and 'ill', eg, 'the joke was well intended'. However, constructions such as 'a well-intentioned patient' do require a hyphen to avoid possible ambiguity. Retain hyphens for post-pandemic and pre-pandemic

Avoid the overuse of hyphens. Aim to preserve readability.

General rule: if the stylebook doesn't give a ruling on a word that has a hyphen, check in Chambers dictionary and use the spelling given in the dictionary.

When to use hyphens

Adjectivally

Examples: 'A man-eating shark'; 'A blue-green sea'.

Exception

Do not place a hyphen between a number and its unit if the unit is abbreviated:

Example: 'a 2-m piece of string' is wrong; it should be 'a 2 m piece of string'.

However, if the unit is spelt out then add the hyphen:

Example: 'a 2 litre vial' is wrong; it should be 'a 2-litre vial'.

Other instances of hyphen usage

When there is likely to be ambiguity or confusion: re-creation (when creating something again is meant); pre-dating.

Co-publish, co-publication should always be hyphenated.

In-hospital should always be hyphenated if it's being used adjectively, eg, in-hospital mortality.

on-site is always hyphenated

For Greek letters in enzymes and chemical names: For example: g-glutamyltransferase, but chains b thalassaemia. 5-hydroxytryptamine and terms such as C-terminal and N-terminal.

Words with two contiguous vowels: re-elect; salpingo-oophorectomy. Exceptions: coordinate, cooperate, triiodothyronine, pentaacetic acid, tetraacetic acid.

Between two contiguous 't's (post-test) and two contiguous 'r's (inter-rater, under-recognised, over-represented).

Note:

Post implementation - two separate words

Preimplementation - should contain a hyphen, ie, pre-implementation

Adjectives ending in 'like' (jelly-like mucus); 'style' (1930s-style equipment); 'type' (Braeburn-type apple); 'wide' (city-wide project); and 'free' (smoke-free zone).

For multi-adjectival words try to reword the sentence, or use your judgement as to whether or not to hyphenate.

BMJ Journals style guide and references

Lists

Either use bullet points (preferred) OR numbering/lettering (not both). Use '1. 2. 3. etc' or 'A. B. C. etc'.

All lists should start with capital letters.

All lists should have full points at the end of each sentence.

Run-in lists: (1), (2), (3),... 'or' (a), (b), (c),...

The only exception to this is if the author requests different styling after they have seen the typeset proof and then it is up to the Production Editor's discretion if the change should be accepted.

BMJ Journals style guide and references

Medical terms (spellings and preferences)

Alzheimer's/Alzheimer

Leave as author supplies but be consistent within the paper (one or the other, but not both).

Amino acid substitutions

Use '>' Do not use arrows (→). For example: '171G>C' (not: '171G→C')

B cell, B lymphocyte - B = bone marrow. Do not spell out.

B₆, B₁₂ (vitamins) (the numbers should be subscripted) - Not 'B-6', 'B-12'

bid, tds, qds, qd

Change 'bid' to 'two times per day'; 'tds' to 'three times a day'; 'qds' to 'four times a day'

Note: 'qd' means once per day.

Birth weight—not 'birthweight' Exception: 'low birthweight babies' is correct

Blinding (in trials)

open; unblinded; non-blind(ed) are all OK, but prefer non-blinded; prefer blinding to masking

Blood groups

Rh positive blood Rh haemolytic disease AB+, O+

Blood groups: the + or - symbol is on the line and close up

Blotting—Southern, northern, western

Only 'Southern' is capitalised as it is named after someone

Bradycardia—not 'bradyarrhythmia'

Breastfed, breastfeeding, breastfed infant, breastfeeding mothers

C1, C2, C3 . . . C7

First, second, third, etc (cervical vertebrae)

C1 2 disc

C2 3 disc, etc, disc of cartilage between first and second vertebrae, etc

C3, C4, C1q

Components of the complement pathway

C reactive protein—not 'carbohydrate reactive protein'

C-terminal

Candida, candidiasis—not 'Monilia', not 'moniliasis'

caesarean

Caregiver—Not 'care giver'

BMJ Journals style guide and references

creatine kinase—Not 'creatinine kinase' or 'creatine phosphokinase'

case notes—two words

Check-up—hyphenated

Chickenpox—one word

Childbearing—one word

Cholelithiasis—not 'choledolithiasis'

CD4 cells CD4:CD8 ratio

Retain + or - symbols as supplied by author. Minus signs should be made on the line and not superscript.

chromosomes: eg, 46,CC and 46,XY

Always use Arabic numbers when numbering chromosomes.

Ciclosporin—not 'cyclosporin'

coenzyme A

Not abbreviated to 'coA'

coagulation factors

Coagulation factors should be in Roman numerals; platelet factors take Arabic numerals.

cognitive-behavioural therapy—not 'cognitive behaviour therapy'

complement is always abbreviated to C1, C2, C1q, etc

C peptide—not 'connecting peptide'

cyclo-oxygenase-2 inhibitors—not 'coxibs'

Cut-off—hyphenated

Death rate/mortality rate—leave as author supplies

Down's syndrome or Down syndrome—leave as author supplies

Enzymes

Enzymes are hyphenated, eg, 'catechol-O-methyltransferase'

Fetus—not 'foetus'

'growth retarded fetus' is OK

fever, feverish—not 'pyrexia' or 'pyrexial'

Fibrillar—not 'fibrillary'

Influenza—not 'flu'

Follow-up—one word and hyphenated

BMJ Journals style guide and references

Funduscopy—not 'fundoscopy' (this is not the same as 'ophthalmoscopy')

Gastro-oesophageal—not 'gastroesophageal'

Blood cell counts

g/L—haemoglobin units (formerly g/dL, please use g/L now)

See the table below for white and red blood cells counts, taken from The Association for Clinical Biochemistry & Laboratory Medicine (<https://www.acb.org.uk/>) -

Recommendation for Full Blood Count

Analyte name and abbreviations to conform to the National Laboratory Medicine Catalogue (NLMC)

Pathology Harmony Proposal	
Analyte	Units
White blood cell (WBC) count	$\times 10^9/\text{L}$
Neutrophil count	$\times 10^9/\text{L}$
Lymphocyte count	$\times 10^9/\text{L}$
Monocyte count	$\times 10^9/\text{L}$
Eosinophil count	$\times 10^9/\text{L}$
Basophil count	$\times 10^9/\text{L}$
Nucleated red blood cell (NRBC) count	$\times 10^9/\text{L}$
Red blood cell (RBC) count	$\times 10^{12}/\text{L}$
Haemoglobin (Hb)	g/L
Haematocrit (Hct)	L/L
Mean cell volume (MCV)	fL
Mean cell haemoglobin (MCH)	pg
Mean cell haemoglobin concentration (MCHC)	g/L
Red cell distribution width (RDW)	%
Platelet (PLT) count	$\times 10^9/\text{L}$
Reticulocyte (Retic) count	$\times 10^9/\text{L}$

Genes

If a human gene is being discussed, the words are italicised and the first letter of each word is capitalised.

In other organisms, only the first word is capitalised (or no words are capitalised) depending on the type of organism.

This is very important, as, for example, the capitalisation in the title immediately signifies whether the study concerns lower organisms or human patients.

When the protein that the gene encodes is being discussed, the name of the protein is not italicised.

Genes

Leave gene names as provided by the author (italic or non-italic typeface), but query inconsistencies within a paper.

H2 blocker, H2 receptor, H2 receptor blocking agent—not hyphenated

H1, H2, etc—antigenic determinants of haemagglutination antigen

Healthcare—one word

History

BMJ Journals style guide and references

A patient can 'give a history' or 'have a history' (but not a 'past history'); 'prior history' or 'previous history' is rarely necessary.

IgA, IgD, IgG, IgM—immunoglobulins A, D, G, M—use symbols without explanation.

IgA1, etc

Subgroups—use subscript

Intramuscular, intravenous—always spell out in full; do not use their abbreviated terms 'im', 'iv', etc.

infectious hepatitis—not: 'infective hepatitis'

interferon , interferon -1b (with hyphen) (IFN-1b closed up).

Use the symbols; eg, inteferon b

Interleukins (IL-1, IL-2, etc)—Interleukin 1, interleukin 2, etc can be abbreviated to 'IL-1', 'IL-2' after first mention.

Leucocyte, leucopenia—not: 'leukocyte'; 'leukopenia' - these are American spelling

Leukaemia

Leucocyte count— same as 'white blood cell count' (usually expressed as ' $\times 10^9/L$ ')

Leukotriene— not 'leucotriene'

Leukoplakia

Lithotripter— not 'lithotriptor'

Live birth— two words

Lupus erythematosus— Not 'lupus erythematosis'

Luteinising hormone— not 'leuteinising'

Monoacetylmorphine—not 'monacetylmorphine'

Monarthritis—not 'monoarthritis'

Mpox—this is the new name for Monkeypox. Following a series of consultations with global experts, WHO has begun using the new preferred term “mpox” as a synonym for monkeypox. As per WHO guidance, mpox should be defined as monkeypox in brackets, ie, mpox (monkeypox) for one year (beginning 29/11/2022) while “monkeypox” is phased out.

Monozygotic—not 'monozygous'

Neuron—not 'neurone' Note: the "neuron" spelling requirement does not apply to the *Practical Neurology* journal, "neurone" should be retained for this journal.

non-Hodgkin's lymphoma—not 'non-Hodgkin lymphoma'

PaO₂—Not 'partial pressure'

phase I, phase II, phase III—phases of trials in development of medicines - use roman numerals

vial—Not 'phial'

Physician/doctor—Usually means a medical specialist. Use 'doctor' unless there is a reason not to (eg, it's distinguishing a physician and a surgeon) — if unsure, leave as author supplies.

Phytoestrogens—not 'phyto-oestrogens'

BMJ Journals style guide and references

Portacaval—not 'portocaval'

Pre-diabetes—hyphenated

Predilection—not 'prediliction'

Pre-eclamptic—hyphenated

Preputial—not 'prepucial'

Presbycusis—not 'presbyacosis'

Primary care physician—term used by non-British writers of non-British healthcare; leave as supplied by author. For the British situation, use general practitioner.

Primers—leave as author supplies, but change apostrophe to prime symbol

Proctoscope—Not 'rectoscope'

Proctoscopy—Not rectoscopy

Progestogen—Not 'progestagen'

Pruritus—not 'pruritis'

Sensorimotor, sensorineural—Not 'sensory-motor'

Sinoatrial—not 'sinuatrial'

Sonographic—not 'sonographical'

Sonorous—not 'sonerous'

Stillbirth, stillborn—one word

Specialty—not 'speciality'

ST segment—not 'S T segment'

Systemic lupus erythematosus—not 'systemic lupus erythematositis'

T helper cells—not hyphenated. Can be abbreviated to, eg 'Th1, Th2 response' after first mention.

TNM classification of cancer

TNM classification of cancer (T, extent of primary tumour; N condition of regional nodes; M, presence or absence of distant metastases). Spell out first time as tumour, node, metastases (TNM) classification, or give explanation in parentheses if just designations are given. Numbers after the letters indicate different degrees of extent: T1 T4; N1 N3; M0 M1; p=pathological, c=clinical (pT3pN1pMX); Tis = carcinoma in situ.

Toxigenic and toxigenicity—not 'toxicogenic', not 'toxicogenicity'

Trial registration number

Change 'trial number', 'trial registry number' etc to 'Trial registration number' and include at the end of Abstracts.

It should be displayed as follows:

BMJ Journals style guide and references

Trial registration number NCT01630863.

Do not insert spaces within the trial number. For example, 'Trial registration number: Australian New Zealand Clinical Trials Registry (ACTRN12611000532909).' is correct.

tube feed, tube feeding, etc

tube fed tuberculous (not 'tuberculose') infection

tuberculous disease (ie, in tuberculosis)

tuberculous leprosy vs lepromatous leprosy or intermediate leprosy

tumorous—not 'tumoral'. Note: 'intratumoral' and 'antitumoral' should not be changed to 'intratumorous' or 'antitumorous', this is incorrect.

type I hyperlipoproteinaemia—also type II, type IIa, type IIb, type III, etc

type I-IV hypersensitivity reactions—Roman numerals

white cell count or leucocyte count (usually expressed as $\times 10^9/l$)—not 'leukocyte' or 'white blood cell'

X linked—No hyphen

X-ray—hyphenated and use capital 'X' (even in the middle of sentence). Typesetter: do not treat X-ray as a fixed lower-case letter like p value.

BMJ Journals style guide and references

Microbiology explanations

Italics and uppercase when describing species and diseases

	Formatting	Example
Family	Not-italic, uppercase first letter	Enterobacteriaceae
Genus*	Italic, uppercase first letter	<i>Escherichia coli</i>
Species	Italic, lowercase	<i>coli</i>
Disease	Not-italic, lowercase except when used at start of a sentence	gonorrhoea
*When several species from the same genus are being, or the same species is mentioned repeatedly, then the genus is written in full when it is first used, but may then be abbreviated to an initial for the following instances. For example, a list of members of the genus <i>Canis</i> might be written as ' <i>Canis</i> lupus, <i>C. aureus</i> , <i>C. simensis</i> '		

Binomial name	Genus	Abbreviation after first use*	Disease†	If abbrev. further#
<i>Trichomonas vaginalis</i>	<i>Trichomonas</i>	<i>T. vaginalis</i>	trichomoniasis	TV
<i>Neisseria gonorrhoeae</i> ‡	<i>Neisseria</i>	<i>N. gonorrhoeae</i>	gonorrhea	NG
<i>Chlamydia trachomatis</i>	<i>Chlamydia</i>	<i>C. trachomatis</i>	chlamydia	CT
<i>Enterococcus faecium</i>	<i>Enterococcus</i>	<i>E. faecium</i>	(neonatal meningitis)	EF
<i>Mycoplasma genitalium</i>	<i>Mycoplasma</i>	<i>M. genitalium</i>	<i>M. genitalium</i> infection	MG
<i>Treponema pallidum</i>	<i>Treponema</i>	<i>T. pallidum</i>	Syphilis	TP
*Use non-breaking space between two parts. Full point after the initial character. †Only capitalise first letter if used at the start of a sentence, otherwise lowercase ‡Notice the 'e' at the end of this. It is commonly missed out. However, the extra 'e' does not appear on the name of the disease. #Only if the author wants to abbreviate this further. Not italic, all uppercase.				

Plural and singular when describing shapes				
Shape/plural	What is it?	Example	Plural	Adjective
coccus/cocci	spherical-shaped bacteria	gonococcus	gonococci	Gonococcal
		pneumococcus	pneumococci	pneumococcal
bacillus/ bacilli*	rod-shaped bacteria			bacillary
*Not to be confused with <i>Bacillus</i> the genus.				

sp and spp

If you want to refer to a species but you do not know what type it is, follow with 'sp', eg, the disease was caused by an *Escherichia* sp

BMJ Journals style guide and references

If you are discussing several species use 'spp', eg, They identified several *Escherichia* spp in the sample Non-italic and always lowercase.

No full stop unless at the end of a sentence.

BMJ Journals style guide and references

Money

\$A	Australia		Chinese currency – use the yuan symbol (¥) – hold down Alt and type 0165 on the number keypad.
\$C	Canada		
Kr	Denmark		
€	Euro		
£	Great Britain		
Kr	Norway		Kroner – specify country on first mention if there is likely to be confusion (eg, Danish kroner, Swedish kronor, Norwegian kroner, Icelandic krónur). Note singular Danish krone, Swedish krona, Norwegian krone, Icelandic króna.
Kr	Sweden		yen, roubles, forints and rupees – spell out in full.
SFr	Switzerland		Swiss francs – use SFr (not Sw Fr). Note position of amount: SFr200; 50 000 forints.
US\$	United States		\$2m or \$2 million? – prefer \$2 million, US\$2–3 million. Do not spell out dollar, change to \$. Change USD to US\$. US\$ – Add US with \$ (but use \$A or \$C for Australian or Canadian dollars).

BMJ Journals style guide and references

Names and addresses

Author names to be left as supplied. No full point after author initials.

Email ID required for corresponding author only. Other authors only require department, institution, city and country for their affiliations.

Author order

Any requests for a change to the author order should be sent to the Editorial Office where the editorial assistant will obtain the necessary permissions from the authors and the Editor of the journal. All of this needs to take place before any amendment to authorship is granted. Do not make changes to the author order without these approvals.

Addresses

Do not translate foreign addresses or anglicise US spellings

Delete all telephone and fax numbers.

No comma between city/county and postcode in UK addresses.

US states should be spelt out, unless the Zip code is present (eg Baltimore, Maryland, USA in an affiliation; Baltimore MD 78009, USA in a correspondence address).

Correspondence address

Correspondent(s) must have an email address (in order for proof to reach them). This should follow the address, preceded by a semi colon.

Corresponding addresses –UK rather than the region, ie, no need for Wales, Scotland, Northern Ireland.

Example:

J Bloggs, Department of Emergency Medicine, Southampton General Hospital, Leaf Lane, Southampton S032 7PN, UK; jbloggs@southampton.co.uk

Affiliation addresses

Example: Department of Emergency Medicine, Southampton General Hospital, Southampton, UK

Authors are not required to provide a department so do not raise an author query requesting a department.

State/province details are optional for all countries **apart** from Australia, Canada and the USA. These must be provided. If authors supply them for large countries such as India and China, etc then they can be retained. They are not required for smaller countries such as European countries such as the Netherlands or the UK.

Important: In affiliation addresses, the United States should always be listed as USA, United Kingdom as UK and United Arab Emirates should be UAE. (If they are not listed in this way they will fall into validation check at upload for Online First stage in Kriya.

An author is listed at the affiliation where the work was carried out. (Corresponding authors can use their current address for their correspondence address if it is different from where work was carried out.)

If an author requests their 'present address' be included it can be listed in the end matter and should be styled as:

Present affiliations The present affiliation of <author name> is: <present address>.

BMJ Journals style guide and references

Author(s) and affiliations

Single author

Where only one author is present there is no requirement to separate the affiliation from the correspondence address. In hybrid journals we only need to include the **Correspondence to** line (because it already includes an affiliation address). On OA titles we need to include the affiliation, because the **Correspondence to** line only lists an email address.

Author titles

Dr, Professor, Assistant, Associate etc, allowed for correspondent (Professor, Associate, etc should be spelt out).

No full point after 'Dr'.

Remove author titles (eg, Dr, Professor) from author lines.

Author initials

Initialising authors' names: McClain Simpson (MS), Robert van Gough (RvG), Tjalling van der Schors (TvdS), John De Niro (JDN), Elise D'Huart (ED'H)

Authors with identical initials

When more than one author has the same initials, it should be the first initial plus two letters of the last name.

Author qualifications

Remove all qualifications from author details (eg MRCP, MD)

Author status

The following authorships are recognised and can be tagged in Kriyadocs

Equal contributor

First authorship

Senior authorship

If an author requests joint last authorship this should be tagged as Senior authorship as they are the same thing.

Asterisks in author names

We DO NOT use asterisks next to authors' names to denote authors contributing equally or for joint senior authors.

Correct

J Brown, R Black, B White

RB and BW contributed equally [this would be in the gutter]

Incorrect

J Brown, R Black,* B White*

*These authors contributed equally [this would be in the gutter]

Deceased author(s)

We do not use the "dagger" symbol to denote any author who has died. A note can be added in the gutter on the first page.

Please use the author's initials not their full name, ie, John Smith would be JS. Do not include the date they died.

If there is more than 1 deceased author note that the wording in the gutter does not need to be placed on separate lines.

Use initials for the authors and do not include dates they died. Note punctuation and style in the example below:

"BH deceased; TAK deceased"

Note to production editors:

Circumstances in which an author in a prominent authorship role has died after publication

In the event that an individual has died after the publication of the work, BMJ does not typically amend details of the corresponding author or guarantor. If in the view of the authors there is a need to update details for correspondence, a rapid response or post PAP correction can be published.

Authorship responsibilities: The deceased author cannot be the submitting author, corresponding author, or the work's guarantor, at the time of publication. If necessary, these roles should be reallocated among the living authors.

BMJ Journals style guide and references

Affiliation address - updating

The affiliation address should be the one the author was affiliated with when the study was carried out (NOT their new affiliation, if they have since moved).

Collaborators

These are persons or groups to be listed on PubMed other than authors and who have made significant contributions to the research or study but may not meet the formal criteria for authorship (such as intellectual contribution or responsibility for content). Collaborators are typically acknowledged for their work in areas like data collection, technical assistance, or providing specialized expertise.

The position of the collaborator group in the author byline should be retained as per the original manuscript and SC1 XML. The pre-editors will usually take care of tagging the collaborators, however, they can be inserted at the Publisher Review stage.

Contributors

A list of which author did what, for example: 'JB collected patient data, RB provided the graphs...'

Equal contributorship should be tagged in Kriya so they appear either in the gutter on the first page or in the end statements.

On Behalf Of:

This refers to the collective authorship of a group or consortium, where individual authors submit the paper on behalf of the entire team or organization. The phrase "On behalf of" should always be placed at the end of the author list, followed by the name of the group or consortium. This indicates that the individual authors are submitting the paper as representatives of a broader group, which may include contributors from multiple institutions or teams.

See this example:

<https://fn.bmj.com/content/early/2025/03/04/archdischild-2024-327894>

If authors provide further information in the On behalf of statement then this information should be moved to the Contributors section in the end statements.

Acknowledgements

People/institutions to be thanked.

Other 'gutter' items:

If authors include the below information, it should go on the first page, in the gutter (above the Acceptance date):

An author has died.

A note about authors contributing equally.

A note about the data / paper having been presented elsewhere (usually a conference).

Manufacturers

On the first occurrence in the text, if the address is provided by the author then it should be presented by Name, City, State (US only), Country. No need to AQ if not provided. Thereafter only name is required.

Do not change organisations' official names – eg, do not add a comma to 'Leigh, Day & Co', and leave the ampersand BUT do remove 'Ltd', 'plc', 'Inc', etc.

Patients

We don't use names or initials to identify patients, eg Anna H or Mrs H. Change to a phrase like "a 64-year-old woman".

Patient consent statement must be present if individual cases are mentioned.

Other

Telephone numbers only used in Notices and some fillers. Abbreviate telephone to Tel, and likewise for Fax, followed by colon. Always supply international country code complete with '+'

For London numbers, the 7 and 8 are separate from the 020 and part of the next group of numbers: For example: 020 7383 6418 (International format: +44 (0)20 7387 4499)

BMJ Journals style guide and references

Numbers and units

Ages

Use numerals for ages and use adjectival hyphens; eg a girl aged 12 years; a boy aged 12 months; a 3-day-old child; a 64-year-old man; men in their 60s; a man of 60.

'60s [1960s]; 60s [age]

33rd week of pregnancy

Coagulation factors—use Roman typeface.

Decimal fractions—precede with a zero: eg, 0.16 (not .16).

Decimal places—Be consistent in the use of decimal places. Round up if too many (except for p value). Example: 0% (not 0.00%).

Degrees—degrees Celsius (°C) - close up to number; eg, '41°C'.

Inches—only use inches in non-scientific articles (obituaries, personal views) and only when it would be nonsensical to use metric units; spell out; don't abbreviate to "in".

IU—international unit.

Measurements—microlitre— μ l miles per hour, mph minuscule (not miniscule) mm Hg (millimetres of mercury), not torr.ml/kg/min not ml kg⁻¹ min⁻¹.

n (number)—'No' is the usual abbreviation for number. 'n' used within parentheses to indicate, for example, the number of observations on which a calculation is based; eg, 'p<0.001 (n=183)'; it is also used in tables.

Per cent—Two words. Exception *Injury Prevention* and *JNIS* where it is one word.

Punctuation in numbers—Think about the punctuation for ease of reading. This is awkward: mean change 3.7, 4.7 to 2.7, vs 2.5, 1.7 to 3.4, P=0.06. This is better: mean change 3.7 (4.7 to 2.7) vs 2.5 (1.7 to 3.4), P=0.06.

Platelet factors—Use Arabic numerals.

Times—Leave as supplied by the author. However, please ensure it is consistent throughout.

SD, sd, S.D., s.d.—Change all to SD

Four-figure numbers and upwards—Close up four-figure numbers (4000). Use a thin space in place of commas in numbers over 9999 including in tables, to divide into groups of 3: so 10 000; 125 000 000.

L /mL For litres/millilitres use L / mL, respectively.

Numbers less than 10—Spell out numbers less than 10 that stand alone (ie, are not a unit or measurement): Example: 'Only two papers involving 25 patients'

Numbers at the beginning of a sentence—There is no need to spell out numbers at the beginning of sentences (ie, 1445 cases... does not require spelling out as one thousand four hundred and forty five cases).

Compound numbers should be hyphenated.

BMJ Journals style guide and references

In structured abstracts and tables, a sentence can start with a number—this number is not followed by a capital letter.

Mathematical symbols—Mathematical symbols should be closed up to the number; a thin space will be added at page proof stage.

Number spans—Use en rules to indicate a range. Exception: CIs or when + or - signs are used (then use 'and').

Numbers in lists—The sentence below is effectively a list; no need to spell out the numbers under 10: Example: During the first survey, other skin infections included abscesses in 5 (8%) children, fungal infections in 15 (25%) children and uncomplicated scabies in 3 (5%) children.

Numbers and percentages—OK to use just percentages in discussions, rather than reiterating numbers. The principle is that numbers must be given somewhere, but if they are given in a table and the text just highlights some of the data, it may be legitimate to give just the percentage.

It's usually not necessary to use percentages for numbers in a single group, or when numbers are 100 or less. But percentages are useful for comparisons.

When some denominators are over 500 and others under, it's OK to mix whole percentages with percentages to one decimal point, eg, in separate columns of a table. The concept of significant digits may be useful here.

Page numbers—Page numbers in references should be as succinct as possible: Examples: 88–9, 414–17, 1399–401.

P values / p value / p values

Please ensure you include the word 'value' when referring to p value. It should never be just "P".

In tables, the header should always be P value (cap P and singular).

In text, if p value is at the start of a sentence then it should be P value (or P values if plural). The exception to this rule is in footnotes, where they should all be lower case, eg, $p < 0.05$, $p < 0.01$, $p < 0.001$.

If it's not at the start of a sentence it should be p value (or p values if plural).

Rounding up numbers—Blood-clotting factors take roman numerals, platelet factors arabic numerals.

Use units (not pints) when referring to quantities transfused, so do not convert to litres. Counts: red cells, 10^{12} ; platelets, 10^9 ; white cells, 10^9 ; should be per litre (not μL , ml or mm^3).

mm Hg (thin space in the middle) 60% (45% to 70%) 45 mg/ml (20 to 50 mg/ml)

Scores—Use numbers for scores on tests and scales, ratios

twofold, sixfold, 2½-fold, 2.5-fold, 18-fold

Units—Abbreviations and symbols must be standard and SI units used throughout, except for blood pressure values which are reported in mm Hg. The metric system is preferred for the expression of length, area, mass, and volume. Note: specific style in *Journal of Clinical Pathology* (JCP)—Haemoglobin concentrates should be expressed as g/L rather than g/dL. (As of 14/09/2018, this style is followed across all journals.)

Units should always be used within ranges, for example, $0.01\text{mg}/\text{m}^3$ to $0.10\text{mg}/\text{m}^3$, 10cm to 20cm, 10 m to 20 m, etc.

However, when an en rule is used, the unit does not need to be added to the first number in the range eg, mean 35 min, range 15–68 min

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EXCEPTION: DTB

Drug doses in DTB do not have a space between the number and the units (25mg NOT 25 mg)

BMJ Journals style guide and references

Other points of style

Acknowledgements

This is free text written by the authors. Personal pronouns are allowed, no need to change them to third person. Copy editors should always check the grammar and spelling in the acknowledgements section and correct any poor grammar or spelling.

Bold font, italic and underline

We only use bold typefaces in standard places, such as, ABSTRACT, Correspondence to, subheads, Table 1, table headers, Figure 1, etc. Bold typeface should not be used for emphasis in the middle of a paragraph. Instead, if an author wants emphasis, use italics.

Italics should be removed for statistical terms, numerical systems and notations.

Italics should not be retained for terms and notations such as "MZ," "Z," "G," "ET," "P(G)," "DZ," "(P(G) = m/N)," "P(MS|ET)," "Zw=P(MS|G, F, ET)," and so forth.

Italics should be used for equations.

Correspondence article titles

For correspondence article titles, the title should be changed by the copy editor in most cases, taking the details from the citation to the original article in the references list (usually the first reference). The PE will then check this is correct at the Publisher Review stage before signing off.

"Correspondence on "<original article title>" by <author name et al>"

(where author = author of original article)

If there is a response to a Correspondence piece, the format is:

"Response to: Correspondence on "<original article title>" by <author name et al>" (where author = author of correspondence)

Standalone correspondence

The titles for these items should **not** be edited. These are standalone letters and are not commenting on other content in the journal, or responding to a published item. Treat them as standalone correspondence (example [here](#)).

Letters

Greetings at the start of letters, such as, Dear Editor, Dear Sir/Madam, Sir, Madam, etc should be removed.

No heading is required for letters either, eg, "Research letter"

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Cite as: Bennett PM. *BMJ Mil Health* 2022;;1–1. doi: [military-2022-002236](#)

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AO Letter to the Editor,

BMJ

The Combined Services Orthopaedic Society was formed in 1974 to promote orthopaedic specialty training. Academic meetings are held annually, hosted in turn by each of the Services. The meetings are open to those with an interest in Trauma and Orthopaedics. In addition, Consultant Advisors to the Defence Medical Services whose work has significance or relevance to the military are invited. Trainees present on a broad range of orthopaedic topics during the academic component, which is followed by a formal Mess Dinner.

The meeting in 2024 will represent the 50th Anniversary of the Society. Our President, Mr Simon F. RAMC, are keen to have as many current and past members of the Society in attendance as possible to like to attend. We are invited to contact secretary@csos.org.uk for further information.

Underline

Underlining of words and letters is allowed although we would like to avoid underlining text if possible. Sometimes authors use it to denote the first letters of an organisation.

End statements

The following statements are free text for authors so production editors should always check the following end statements: acknowledgements, contributors, competing interests and ethics approval. They should be checked for spelling, grammar, punctuation and sense. Also, check funding statements are inserted correctly, ie, not in the acknowledgements section.

Foreign phrases—do not italicise.

male and female

Use 'man'/'woman' or 'boy'/'girl' (up to and including age 16) when this is meant. Children up to and including age 16 and referred to as male/female etc should continue to be referred to as either male/female or boys/girls, do not change to 'men'/'women'.

Use 'male'/'female' adjectivally only (not as noun). If the data set includes boys and men then you would use males as a noun and for girls and women then females, for example "10 males ranging in age from 12 to 25"

Rather than 'male and female suicides' it is preferable to say 'men and women who committed suicide'. If children are included, then you may use 'males and females . . .' if necessary.

Copyright © , Trademark ™ and registered trademark ® symbols

TM and Registered TMs are not permitted anywhere in the article (apart from in the references).

Articles should not include Trademark™ or Registered® trademark symbols in the main text, tables or figures.

Delete any copyright symbols that appear after product names in authors' manuscripts, but retain the capital letters if they have been used, eg, Calpol, Panadol.

Authors should be asked to remove them from their figures if they are included. If authors require further explanation for this style point, PEs can advise: "by including them we risk rights being asserted that do not exist, and so BMJ's position is to eliminate them entirely"

This editing should NOT be carried out if any of these symbols appear in the references - © , ™ and ® symbols should be retained in reference titles if they have been included in the original publication (ie. as per the pubmed/crossref citation).

BMJ Journals style guide and references

If authors request ™ to be reinstated at the proofing stage this is fine. However, do not reinstate ® symbols - if the " ® " was applied to a mark which is not registered that could be serious as rights would be being asserted that do not exist (which is an offence).

Textual footnotes

Most of our journals do not allow footnotes and these should be avoided where possible. For textual footnotes use lower case, superscripted Roman numerals (i. ii. iii).

Utilise—Change to 'use.'

"utilising" should be changed to 'using' and 'utilised' to 'used'.

However, do not change utilisation to usage. They are similar nouns but have different meanings.

'Arrows' in amino acids—Use '>' rather than an arrow.

'>' indicates a substitution at DNA level (eg, c.76A>T).

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Patient consent for publication

1. Obtained

If CD value is 'Yes'

2. Parental/guardian consent obtained [for children <7 years old]

If the CD value is 'Yes' and the explanation specifically mentions parent/guardian consent.

3. Next of kin consent obtained (this is already an option on SC1 and will transfer via xml if selected at submission). This is rare.

If the CD value is 'Yes' and the explanation provided states next of kin consent.

4. Not required.

If the CD value is 'No' and you are not sure about the explanation provided please use "Not required" and raise a query to the Production Editor.

Note: If the CD value is 'No' and you are not sure about the explanation provided please use "Not required" and raise a query to the Production Editor.

We will NO LONGER be using the statement 'Detail has been removed etc' - if a patient is sufficiently anonymised it will be 'Not required'.

Deceased patients - if we cannot get a relative's consent/next of kin then we assess if info can be removed to anonymise the paper. In which case the options will be 'Next of kin consent obtained' or 'Not required'.

Patients who lack capacity - we do not usually accept consent on behalf of someone in this case, so again the paper would need to be anonymised. Options here would also be 'Next of kin consent obtained' (very rare) or 'Not required'.

Participants vs patient consent.

If an author mentions that they obtained consent from participants in a study then this is different to obtaining consent from patients. Leave this sort of text within the body of the article and do not move it to the end statements. Also, the patient consent statement at the end should not be altered from what is provided in the Scholarone output.

****IMPORTANT****

We DO NOT publish patient consent forms (i.e. forms that contain sensitive personal data (such as date of birth, email address, etc)). Please be mindful of this requirement when reviewing/selecting files from both the 'doc' folder and the 'suppl_data' folder from the input files, and note that such files may not always be labelled as 'patient consent' and could be named anything at all. If there are any doubts AT ALL please alert the production editor (via email with prodmanager@bmj.com in copy) ASAP. Please use subject line **'Red flag - possible patient consent issue'**.

Pre-edit team to check XML file designation tag if citations within manuscript do not accurately match with files exported.

Permissions

If you read a table or figure legend that mentions the item is from another source, and there is no mention of permission, can you please query the author asking if they have permission, and if so, that they amend the table/figure legend accordingly to make that clear.

It is important when using other works that copyright is protected—we would expect other publishers to respect our rights and this must be reciprocated. We have an agreement with the STM Publishing Association whereby we may reuse limited amounts of content from the publications of other members free of charge and without the need to obtain permission from the publisher or via Rightslink. Information on what may be done with this content and who is signed up to the agreement can be found here: <http://www.stm-assoc.org/permissions-guidelines/>.

When we reprint an illustration, table or appendix we need to ensure that we have permission to do so. The easiest way to acquire permission from most publishers is through Rightslink. The link for this can be found at article level on the publishers website, usually called 'Request Permissions'. This service grants instant permissions. Some publishers may need to be contacted directly, if so allow plenty of time as some publishers may take up to 4 weeks to respond to requests.

Once permission has been granted the work must be reprinted with the proper reference: the format is "Reprinted with permission from "full reference", unless the publisher has requested other text.

Legends for figures—give the author(s) of the reference – (adapted from Venables et al[4]).

Poems—Copyright permission is needed to reprint poems, again via Rightslink or by contacting the publishers. Permission is required for more than 25% of a poem, or 20 lines from a poem, whichever is the less (ie, in a poem of four lines, permission is required to reprint more than one line of it). Titles are italicised or put in quotation marks if cited in italic matter, they do not need quotation marks.

If you wish to adapt a table or figure you will need to approach the publishers of the original work to have the adaptation approved. When you have permission acknowledge that they have been adapted; give reference number in legend.

If you would like any further guidance or information regarding permissions or copyright please contact Laura (llacey@bmj.com) or Julie (jhalfacre@bmj.com).

BMJ Journals style guide and references

Prefixes, suffixes and compound words

anti- (as prefix)

Write words prefixed by anti- as all one word unless it looks odd, eg, with letters (anti-HBs), with names (anti-Bitis-Echis-Naja serum), with words beginning in i (anti-icteric) and with two-word compounds used as adjectives (anti-gas gangrene serum).

-borne (as suffix)

Always closed up (eg, foodborne, waterborne, bloodborne)

cardio- (as prefix)

Never hyphenated, always one word (eg, cardiovascular)

Chemicals and enzymes with a prefix

Chemicals and enzymes with a prefix (number or Greek letter) should have a hyphen –for example, 17- β -oestradiol.

child- (as prefix)

Always close up (eg, childcare, childbirth)

prefixed words

Avoid splitting prefixed words in an effort to abbreviate: eg, say "preoperative and postoperative treatment" rather than pre- and postoperative treatment, "extrahepatic and intrahepatic" rather than extra- and intrahepatic. The following should continue to be hyphenated: Low- and middle-income countries" and "short- and long-term plans"

co-

(as prefix) usually forms one word with whatever follows it (eg, coauthor, codefendant, codominant, coenzyme, coexist, cofactor, coworkers). Exceptions: co-mutagenicity,

co-twins, co-parents, co-founder. Use hyphen with any other unusual word, use judgement

cis- (as prefix in chemical names)

Italics, hyphenated. Exception: cis fatty acids

Counter (as prefix)

Always forms one word

down- (as prefix)

Not hyphenated. Exception, downregulation, one word

-fold (as suffix)

Spell out as one word when combined with whole numbers up to nine (twofold, sixfold, ninefold) and in all other cases insert a hyphen (2½-fold (or 2.5-fold); 3.6-fold;

half- (as prefix)

Always separate word, no hyphen unless adjectival. Exceptions: halftone, halfway.

in- (as prefix)

Not hyphenated (eg, inpatient)

intra- (as prefix)

One word unless it's the prefix of a word beginning with 'a', when it takes a hyphen (eg, intra-abdominal, intra-acinar, intracavitary (not intracavity), intractable).

BMJ Journals style guide and references

heart- (as prefix)

Always spell as two words (eg, heart rate, heart burn)

-like (as suffix)

medico- (as prefix)

Make one word

mid- (as prefix)

Is usually hyphenated. Exceptions: midaxilla, midbrain, midday, midline, midnight, midpoint, midstream, midyear, midbrain. mid-20s, mid-30s, etc (age); mid-1930s, mid-1950s, etc (not mid-'30s, mid-'50s, etc, when referring to dates)

non- (as prefix)

Linked with a hyphen (eg non-event)

off- (as prefix)

Not hyphenated unless adjectival (eg off licence)

over-

As a general rule adjectives, nouns, verbs and adverbs prefixed by over should be one word (overanxious, overconfident—but over-roast, over-rule, etc).

para- (in chemical names)

Use lower case italic p and join with hyphen (*p*-methoxy phenylcarbamoyl).

post-, pre-

Established combinations are generally spelt as one word: postmortem, postgraduate, postdate, preschool, prehospital, postoperative etc. Exceptions: when the conjoined consonants or vowels are the same: pre-empt, post-treatment.

post partum, postpartum

One word when used adjectivally (postpartum bleeding was severe); but two when used predicatively: the mother bled profusely post partum

re- (as prefix)

Used with a verb or noun. Do not insert a hyphen unless the original verb or noun begins with 'e' (eg, re-establish, re-enter, re-enlist; but rearm, rebaptism, rearrangement). When there may be confusion with another word (eg, re-ally with really, re-bound with rebound, re-cover with recover, re-creation with recreation, re-sign with resign, re-treat with retreat) use, for example, ally again, cover again, etc, or hyphenate. Not 'rerefer' - use 'refer again.'

semi- (as prefix)

no hyphen unless the other word begins with i: semi-independent; semirecumbent, semiskilled, semistarvation, semiurban

sub- (as prefix)

one word unless sub basal

self- (as prefix)

all compound words with self are hyphenated, eg, self-esteem, self-service, etc

semi- (as prefix)

no hyphen when used as prefix; one word unless the other word begins with i: semi-independent; semirecumbent, semiskilled, semistarvation, semiurban

BMJ Journals style guide and references

Salt-

all compound words with salt are two words – for example, salt lake, salt mine, salt water

-style (as suffix) have a hyphen exceptions: lifestyle.

trans- (in chemical names)

Italics, hyphen. Exception: trans fatty acids

un- (as prefix)

As a general rule any adjective, noun, verb or adverb prefixed by 'un' should be spelt as one word (unneedful, unobvious, unornamented, etc). Exceptions are combinations that might be confused with other words beginning with un- but not as a prefix (for example, unionised and un-ionised)—avoid confusion by saying, for example, 'not ionised'.

under- (as prefix)

As a general rule, adjectives, nouns, verbs and adverbs prefixed by under should be spelt as one word (underestimate, underpeopled). Exceptions to this rule are listed alphabetically. Use a hyphen in compound words with a double r: under-rated, under-ripe, under-represented.

Up- (as prefix)

Do not hyphenate. Use one word, eg 'upregulation'

well- (as prefix)

Not hyphenated unless adjectival, exception well-being, always hyphenated

-wide (as suffix)

With hyphen, eg 'hospital-wide', 'city-wide', 'office-wide'. Exceptions: 'nationwide' and 'worldwide' are established uses.

BMJ Journals style guide and references

language/word usage

Profanities/expletives/swearing

If any of these are used in direct speech then leave these unchanged. If they are not in direct speech, raise a query to the PE (who will check with the editor).

word/phrase	Do not use
BMJ has a people first language policy which means people should not be defined/described by their behaviour or their condition. In practice, this means we should not say smokers, vapers, diabetics, asthmatics, etc. Instead we should describe them as people who smoke, people who vape, people with diabetes, etc	
acknowledgement	acknowledgement is preferred in British English, acknowledgment in American English.
ageing, ageism, ageist	aging, agism, agist
Ago / back	Do not use 10-years back or 3 months back, etc. Use “10 years ago”. Back is slang and informal. The word “back” is frequently used in Case reports by authors and should be changed to “ago”.
Alcoholic / ex-alcoholic / recovering alcoholic	Alcoholic - do not use 'alcoholic', change to male/man/female/woman with alcohol dependency. Recovering alcoholic - change to 'male/female/person with recovering alcohol dependency' Example, A 53-year-old male with recovering alcohol dependency'
Among / while	amongst / whilst (both are outdated)
anglicising (Latin words)	per se, inter alia, in vitro, ` , ex vivo, a priori, ad hoc, in situ. Latin phrases that have entered normal English should not be italicised (per se, inter alia, in vitro, in vivo, ex vivo, a priori, ad hoc, in situ). Exception: always italicise <i>et al.</i>
bottle fed	bottle feed, bottle feeding, bottle-fed infant (hyphen because it's adjectival)
chatGPT	No need to spell out
compare to / compare with	To compare to is to point out or imply resemblances between objects regarded as essentially of a different order; To compare with is mainly to point out differences between objects regarded as essentially of the same order. For example: Thus, life has been compared to a pilgrimage, to a drama, to a battle; Congress may be compared with the British Parliament. Paris has been compared to ancient Athens; it may be compared with modern London.
depression symptoms	symptoms of depression
epidemiology	In epidemiology the technically correct form is “prevalent/incident cases” and “prevalence/incidence rates” (not prevalence/incidence cases)
<i>et al</i>	always italicise. Can also use coworkers, coauthors and colleagues in running text.
female or woman	Do not use: lady

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heterosexual/homosexual/straight/transgender	Please retain the author's wording. Our previous instruction was do not use: 'Straight', 'Gay', however, please use what the author supplies.
male or female	Try to avoid using lady / gentleman when describing patients. Girls and boys is acceptable for children under 16.
We present a case...	Do not use 'hereby'. 'hereby' is no longer in common usage and is only used in legal documents.
'on examination' or 'after examination'	Do not use: 'upon examination'.
A medical history should be described as 'unremarkable' (not 'non contributory')	Do not use the expression 'non-contributory' to describe a medical history. It should be changed to 'unremarkable'. Example: Change 'His medical history was non-contributory.' to "His medical history was unremarkable."
where	Do not use: 'wherein' or 'hereby' (no longer in common usage and are mainly used in legal documents).
sex worker	prostitute
learning disabilities	Do not use: 'mental handicap'.
Intellectual disability	Do not use: 'mental retardation, mentally retarded'
psychiatric hospitals	Do not use: 'mental hospitals'.
high tech	Do not use: hi tech.
feedback / feed back	feedback (noun); feed back (verb)
follow-up	always hyphenated
fundholding	fundholding (adjective and noun) – one word; fundraising (adjective), fund raising (noun or verb); budget holding (two words, adjective)
interchangeable	not interchangeable
install (not instal)	install (not instal)—but use enrol, fulfil, instil
learned	use learnt (unless scholarly is meant)
manikin / mannequin	manikin—used in teaching anatomy; mannequin—used for displaying clothes
meanwhile / mean time	meanwhile (one word, the intervening time); but, in the mean (intervening) time (two words)
movable	Not moveable as this is US spelling
part time, part timer	Not hyphenated
patient with Down's syndrome	
Use: 'Down syndrome' or 'Down's syndrome' (as provided by author).	Do not use: 'mongol'.
patients with symptoms	Use: 'patients with cancer', 'patients who had a stroke', 'patients who had heart/lung transplants', etc. Do not use: 'cancer patients', 'stroke patients'. <i>Exceptions: transplant patients; trauma patients. COVID-19 patients</i>

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sizeable (not sizable)	
Smokers / vapers	People who smoke / people who vape
wheelchair users	Do not use: wheelchair bound.
a 67-year-old woman	Do not use people's names, eg, 'Anna H' or 'Mrs H.'
Use as one word:	stepwise, throughflow, healthcare, secondhand
the patient	Do not name patients as, eg, 'Mr A', etc.
cipher	Do not use: 'cypher'.
exorbitant	Do not use: exhorbitant.
sulfate	Do not use: 'sulphate'. Use 'f' spelling for all sulfa-type words.
transmissible	Do not use: 'transmissable'.
until	Do not use: 'till' or 'til date'.
committed suicide	change to died by suicide (suicide is no longer a crime). See http://www.bmj.com/content/354/bmj.i4871/rapid-responses
suicides	rather than "male and female suicides" it is better to say "men and women who killed themselves" or "who died by suicide"; but if children are included then you may use "males and females. . ."

BMJ Journals style guide and references

Punctuation

Full points in boxes

If the author writes full sentences then full points should be used.

Parentheses and brackets

Always use parentheses (round brackets) even for parentheses within parentheses.

Note for PEs: Please change square brackets to round brackets if inserted by the author at review stage. If authors insist on square brackets being used after being sent a revised proof for review then it is ok to reinstate them.

Exceptions:

Square brackets around an ionised substance [Ca²⁺] (and concentrations in chemistry).

Square brackets for an interpolation in a quotation: "He [John] was unhappy".

Square brackets to define an abbreviation used in quoted material: "The BMA [British Medical Association] spokesman said..."

Square brackets for maths expressions.

Commas

Do not use a comma before 'and' in lists: eg, tea, coffee, sugar and biscuits, except where the absence of a comma could be misleading: eg, 'the Bishops of London, Bath and Wells, and Lincoln'.

Use commas to surround 'etc': eg, 'tea, coffee, etc, were served.'

Use comma after 'eg' and 'ie' in parentheses -(eg,).

For journals which use American English authors can use the Oxford comma in lists.

Question marks

Question marks—needed only for direct questions. Direct questions included within a sentence: Examples: Lower case after a comma, eg: The question is, who should deal with this? Lower case after a colon, eg: We asked three questions: who? where? when?

Quotation marks

Use single quotation marks—only use double marks for direct speech/conversation. If authors add italics and/or underline for extra emphasis then these should be removed and just double speech marks retained. Should be curly and not straight quotation mark. In Kriya use the Symbols palette to change/insert curly quotation/speech marks.quotations

The screenshot shows the Kriya software interface. At the top, there's a toolbar with icons for undo, redo, bold, italic, text color, background color, bulleted list, numbered list, link, unlink, and a symbols palette (Ω). Below the toolbar, the document title is "European Journal of Hospital Pharmacy" with a URL and article ID. The main content area shows a document preview with a title "Levofloxacin induced bullous fixed drug eruption: a rare case report" and several footnotes. A symbols palette is open, showing various symbols like currency, mathematical, and special characters. The document preview includes footnotes for authors' departments and a correspondence address.

Footnotes:

- ¹Department of Pharmacy Practice, ISF C
- ²Department of Pharmacy, Uttarakhand Te
- ³Department of Pharmaceutical Science a
- ⁴Department of Pharmaceutics, Himalaya

Correspondence to: Dr Armit Sharma, D
Pharmaceutical Science and Technolog
AS and AB contributed equally.
AS and AB are joint first authors.

BMJ Journals style guide and references

After a phrase or word has been introduced in quotation marks it is not necessary to use them again for that word or phrase, except when it is being used subjectively and does not mean what it says: eg, a 'marvellous' reducing diet that in fact increases weight.

Quotation marks have to be used the first time a 'strange' term is used in the Abstract and the first time it is used in the rest of the paper.

Conversation is usually introduced with a comma and the first word inside the quotation starts with a capital (if it is the start of the speaker's sentence). Example: The nurse said, "Two swabs are missing." "Two swabs were missing this afternoon," said the nurse, "but I found them this evening." A new speaker demands a new paragraph.

If you are making the speaker's sentence form part of yours do not use a comma: Example: Reynolds, writing on symptoms, noted that they 'resolve themselves into modifications of structure'. But: Reynolds, writing on symptoms, noted, 'They resolve themselves into modifications of structure'.

No full stop at end of bold quotations (lifted quotes) in personal view, etc.

A direct quotation needs quotation marks. Quotations should be used sparingly and in exceptional circumstances. For long quotations (about 50 words) quotation marks should be deleted and the text given full out, reduced by one point size.

All types of quotations, for example, pull in quotes, inline quotes, etc, have quotation marks.

BMJ Journals style guide and references

Block quotes

Note to typesetter: There should be line spaces between the sections of the quote.

Psychological, pragmatic and social influences on non-attendance

In response to being asked why people might not attend DRS, professionals and patients both acknowledged that denial of having diabetes could contribute. One patient missed screening appointments because she disliked the proximity of the screener. Pragmatic reasons raised by the non-regular attenders for non-attendance included work commitments and postoperative recuperation.

Some people just... have their head in the... like the ostrich, they don't have diabetes or they're not taking any notice of it and they will just... yes, not come. Some because they think they can't have the time off work, you know? Screening Programme 1 (Region 1)

It's just the thought of somebody coming close to my eye.
Patient 15 (Region 3, Non-regular)

I missed once, because I had an abscess in an awkward place, and I had to have an operation. But the following year I made sure. Patient 5 (Region 3, Non-regular)

Another non-regular attender who identified herself as a regular attender had attempted to access DRS via

white

BMJ Journals style guide and references

Racial usage

Ethnic groups

'White' and 'black' are ok adjectivally (eg, 'black adults', 'white patients'), but do not use 'whites' and 'blacks' (as nouns).

Black and minority ethnic (BME), BAME (black, Asian, and minority ethnic), or people of colour?

Follow what the author uses but prefer 'ethnic minority.'

Asian, Hispanic and Latino are all fine to capitalise because it's referring to the place of residence, where a person is descended, or the language they speak, whereas 'black', 'white' etc is referring to skin colour.

'LGBT+' is preferred style but it is fine to retain other variations if the authors request.

'Trafficked people' (avoid victims).

BMJ Journals style guide and references

Respiratory symbols

A-aDo₂ — alveolar-arterial oxygen tension difference

FEV₁ — forced expiratory volume in one second

Fio₂ — fractional inspired oxygen

FIVC — forced inspiratory vital capacity

FVC — forced vital capacity

FRC — functional residual capacity

Gaw — airways conductance

Paco₂ — arterial carbon dioxide tension (or pressure)

Paco₂ — alveolar carbon dioxide tension (or pressure)

Pao₂ — arterial oxygen tension (or pressure)

PC₂₀[PD₂₀]FEV₁ —

provocative concentration [or dose] causing a 20% fall in FEV₁ (a measure of bronchial reactivity)

Pco₂ — partial pressure of carbon dioxide

PEF — peak expiratory flow (sometimes given as PEF_R, peak expiratory flow rate)

Peto₂/Petco₂ — end tidal oxygen/carbon dioxide tension

PO₂ — partial pressure of oxygen

PIF — peak inspiratory flow

Raw — airways resistance

RV — residual volume

SaO₂ — arterial oxygen saturation

sGaw — specific airways conduction

TcPO₂ — transcutaneous oxygen tension

TLC — total lung capacity

TLco — carbon monoxide transfer factor (formerly DLco)

Va — alveolar ventilation

VC — vital capacity

Vd — dead space

VE — minute ventilation

Vemax/Vimax — maximum expiratory (inspiratory) flow

BMJ Journals style guide and references

V_{e50}/V_{i50} — forced expiratory (inspiratory) flow at 50% vital capacity

VO_2 — oxygen consumption

VCO_2 — carbon dioxide production

V_t — tidal volume

VTG — thoracic gas volume

Watch out for confusion with Pa used for partial pressure and pascals. Pa = arterial tension; P_A = alveolar tension.

V = volume; $\dot{V}$ = ventilation.

BMJ Journals style guide and references

Statistics and fractions

Degrees of freedom

Degrees of freedom is abbreviated as 'df'. Do not subscript.

chi square—use Greek letter: χ^2

Confidence intervals (applies to tables and main text)

Abbreviate confidence intervals to 'CI'.

In confidence intervals, use "to" rather than a hyphen or an en rule (and other ranges that may contain negative values); eg, '... (CI -7.0 to 0.9 kPa) ...' Using hyphens or en rule may risk confusion with minus signs,

There is usually no need to repeat units when giving CIs: eg, 19 mmol/l (95% CI 14 to 27). However, when the CI is a percentage, include the % sign: eg, '17% (95% CI 2% to 46%)'.

Occasionally, authors may present data which is difficult to read and the copy editor may need to consider the punctuation for ease of reading, for example,

This is awkward: mean change 3.7, 4.7 to 2.7, vs 2.5, 1.7 to 3.4, $P=0.06$

This is better: mean change 3.7 (4.7 to 2.7) vs 2.5 (1.7 to 3.4), $P=0.06$

If the author uses semi colons rather than commas then they may not be presenting a range of CIs. Consider the context of the sentence before changing anything. If in doubt then leave as the author supplies and/or raise a query to the PE.

Further clarification regarding CIs

If the author has used a comma or semicolon, change them to 'to'.

If the context is not a range, retain the use of commas or semicolons as appropriate.

In cases where the usage of commas or semicolons is unclear or potentially confusing, raise a query with the relevant PE to seek confirmation.

correlation coefficient, r

The symbol 'r' must always be followed by '=', even when the value is negative, eg '(r = (0. 1))'. Do not use Greek letter rho; use r.

F is the variance ratio (*italic*)

Follow as author supplies: eg, $F_{2,24}$.

1/2 (half)—Spell out as 'half' or use '0.5' but use the fraction for ages, eg '8½ years old'. Use words in text: eg, 'a quarter of the population smokes'.

Mathematical symbols

Symbols should be close up to the number; a thin space will be added at page proof stage. Do not replace '±' with '(SD)'. Leave as author provides (although symbol should be ±, not +/-).

NA—NA, N.A., N/A, na, n.a., n/a

NS, N.S., N/S, ns, n.s., n/s

Action: Leave as author provides (but ensure there is consistency throughout article).

NS—Not significant (statistical) is OK in parentheses in text (analogous to p value) without being spelt out.

number, No

Use the abbreviation 'No' only in tables and figures (not in text, table headings or legends); n can be used in parentheses to indicate, for example, the number of observations on which a calculation is based, eg '(n=183)'.

BMJ Journals style guide and references

P values / p value / p values—

Please ensure you include the word 'value' when referring to p values. It should never be just "P".

In tables, the header should always be singular P value.

In text, if p value is at the start of a sentence then it should be P value (or P values if plural).

If it's not at the start of a sentence it should be p value (or p values if plural).

P values must always be expressed as equal to, less than, more than, less than or equal to, or more than or equal to: (eg, $p=0.001$; $p<0.01$; $p>0.01$; $p\leq 0.01$; $p\geq 0.01$). Conventional signs for probability in tables and figures: $*p<0.05$; $**p<0.01$; $***p<0.001$.

Punctuation in statistics

Punctuate so that statistical measures are grouped together; eg: (OR 0.45 (0.24 to 0.85))(OR 0.45; 0.24 to 0.85)(5% (7/149) vs 5% (41/822))(46%; 33 participants) . . . (15%; 11) . . . (11.7% vs 7.0%, $p<0.001$)(21 (7%) vs 4 (2%); $c^2=6.4$, $df=1$, $p=0.01$).

Quartiles

(and other centile-type divisions) are not the interval, they are the line below which a quarter (for example) of the values lies. So use the fraction when referring to the interval: quartile=quarter; quintile=fifth; decile=tenth, etc; say 'in the lowest fourth' or 'on the first quartile (or 25th centile)' as appropriate.

Ratios

Use numerals for ratios; eg, a HDL:LDL cholesterol ratio of 5. Generally ratios are expressed with 'to' or a colon between components (the male to female ratio was 2.7:1.0). Note that the spacing before and after the colon should be the same. A colon can be used in text between words; eg, benefit:risk ratio; cost:benefit ratio; waist:hip ratio; but in narrative 'the male to female ratio' is preferable to 'the male:female ratio'.

Risk ratios

Use numerals (rather than writing out in full). '1 in 10' in text; but '1:10' in tables.

Statistical tests

Use Roman text (not italics). Student's t-test (with a hyphen between t-test). Not 'Student t test'. Mann–Whitney U test.

Statistical terms

Leave as provided by authors (ie, do not change median to mean, etc). However, you should watch out for names of statistical tests that are often spelled and/or punctuated incorrectly:

- Newman-Keuls
- Mantel-Haenszel
- Wilcoxon rank-sum
- Wilcoxon signed-rank
- Mann-Whitney U test
- Kruskal-Wallis
- Kaplan-Meier.

Versus, versus, vs, Vs, vs., VS.

Action: Should be spelt out in narrative text (not italicised), but use abbreviation with numbers in text, or for contraction in parentheses—no full points, vs

BMJ Journals style guide and references

Titles

Omit the opening 'article' words (eg, 'The', 'A', 'An' etc) in titles; however, please DO NOT omit these words in bmjgh going forward (this is a specific request from the Editor).

All manuscripts titles should be in sentence case, eg, "Acute safeguarding essentials in modern-day paediatrics: sexual relationships, consent and confidentiality"

In titles and headings use a lower case letter after a colon (eg, 'Hospices: why delay?').

Use colons rather than em rules in titles and headings. If the author uses more than one colon in a title then these should be handled with an initial colon, followed by an en dash. For example,

Prostate cancer: unmet supportive and palliative care needs [en rule] national survey of patients and family carers

When referring to another section within a paper use the initial cap for the heading and precede with 'the' (eg, 'In the Materials and methods section we described...')

The study design in titles ALWAYS follows a colon and is NOT part of the title (eg, 'Time to pregnancy and sex offspring: cohort study' – NOT: 'A cohort study of time to pregnancy and sex offspring').

Correspondence titles

We have an agreed standard title structure for correspondence articles. Here are the rules for all 'Correspondence' article types

Correspondence on "<original article title>" by <author name et al> (where author = author of original article)

Response to: Correspondence on "<original article title>" by <author name et al> (where author = author of correspondence)

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Useful websites

A guide to English grammar and punctuation.

<http://www.edufind.com/english-grammar/english-grammar-guide/>

Conjugate any English verb

<http://www.verbix.com/languages/english.shtml>

Chambers 21st Century Dictionary:

<http://www.chambers.co.uk/dictionaries.php>

Medical dictionary

Online dictionary which uses US spelling but also gives the UK equivalent,

<https://medical-dictionary.thefreedictionary.com/>

BMJ Journals style guide and references

BMJ Journals reference style

Placement of reference citations

Generally, reference citations should be placed at the end of sentences rather than mid sentence. Exceptions may arise if placing the citation(s) at the end of a sentence may cause confusion to the reader.

Introduction

The emergence of SARS-CoV-2, a novel strain, has transformed COVID-19 into a critical global public health issue. This virus continues to be present in many countries worldwide, posing a significant threat to human health.¹ As per the latest figures released by the WHO, there have been a staggering 760 897 555 reported cases of COVID-19 globally, which have resulted in the unfortunate loss of 6 874 585 lives.² Bangladesh is at much greater risk since it has the highest population density in the world, making concepts such as 'social separation' impossible to implement.³ As a result, it reported 2 037 024 positive cases and 29 439 domestic deaths as of 30 December 2022.⁴ Since SARS-CoV-2 is a highly contagious virus that affects people worldwide, vaccination is the most effective method of protecting the population from COVID-19.^{5–7} Although vaccines may not be 100% effective, high coverage in a community can significantly improve disease control.⁸ Five key factors—access, affordability, awareness, activation and acceptance are crucial in ensuring maximum coverage.⁹

It is also acceptable to place citation numbers after author names if it is citing their study (example 1), however, we would prefer the citation numbers to appear at the end of sentences (example 2).

Example 1:

The article by Tadros *et al*⁶ does an excellent job in exploring how families view decision-making in the paediatric cardiac population. When asked 'who should make this decision?', families overwhelmingly wanted to have near equal input on the variety of decisions for which they were surveyed, including both 'technical' medical situations (eg. starting antibiotics) and 'non-technical'

Example 2

In Gut, Massimino *et al* have reported a novel association of viral infection and IBD.³ The authors have previously reported an upregulation of the HBx protein belonging to the *Orthohepadnavirus* genus, in the gut virome of early diagnosed, treatment naïve

How to cite an online first article

Each Online First article has a unique Digital Object Identifier (DOI). This should be included in the references BEFORE the article

has appeared in an issue, use this citation format:

Ann Rheum Dis 2016. doi: 10.1136/annrheumdis-2016-210152. [Epub ahead of print 28 Nov 2016].

Note: this pertains to References style only and should not be used elsewhere in the paper (eg, 'To cite' details)

See point 44 below for citing other electronic sources

Personal communications

These should be avoided; however, where necessary, only cite personal communications, unpublished data and unpublished conference proceedings in the text, NOT in the reference list.

(F Engels, personal communication, 1992)

(F Engels, unpublished data, 1992)

(N McLaren, Sixth World Conference on Smoking, Perth, 1990)

Articles in journals

Standard journal article

Punctuation of references must follow adapted Vancouver style: 12. Lastname AB, Lastname CD. Article title. *Journal title* Year;vol:start page–end page.

Use one space only between words up to the year and then no spaces. The journal title should be in italics and abbreviated according to the style of Index Medicus. If the journal is not listed in Index Medicus then it should be written out in full. List the

BMJ Journals style guide and references

names and initials of all authors if there are 3 or fewer; otherwise list the first 3 and add *et al.* Except JMG, which lists all authors. The year of the document should be represented in four-digit format.

For example: You CH, Lee KY, Chey RY, *et al.* Electrogastrographic study of patients with unexplained nausea, bloating and vomiting. *Gastroenterology* 1980;79:311–14.

Corporate author (company/institution)

Where a corporate body is chiefly responsible for producing the document, please reference as follows: The Royal Marsden Hospital Bone-marrow Transplantation Team. Failure of syngeneic bone-marrow graft without preconditioning in post-hepatitis marrow aplasia. *Lancet* 1977;2:742–4.

No author given

If no personal author is indicated, use the corporate author (e.g. Tobacco Institute, *New York Times*). If the person or corporate body chiefly responsible for producing the document cannot be determined, it is optional to use “Anon”.

For example: Anon. Coffee drinking and cancer of the pancreas [editorial]. *BMJ* 1981;283:628.

Articles in a foreign language

Please reference in Vancouver style, with article title in the language of origin.

For example: Massone L, Borghi S, Pestarino A, *et al.* Localisations palmaires de la dermatite herpétiforme. *Ann Dermatol Venerol* 1987;114:1545–7.

If English translations of title also provided it can go in square brackets after the foreign language title. Supplementary information can be given in parentheses at end of article title: eg (In Chinese) or (English abstract). If original language not provided, this is ok.

Volume with supplement

Magni F, Rossoni G, Berti F. BN-52021 protects guinea-pig from heart anaphylaxis. *Pharmacol Res Commun* 1988;20(Suppl 5):75–8.

Issue with supplement

Gardos G, Cole JO, Haskell D, *et al.* The natural history of tardive dyskinesia. *J Clin Psychopharmacol* 1988;8(4 Suppl):31–37S.

Issue with part

Edwards L, Meyskens F, Levine N. Effect of oral isotretinoin on dysplastic nevi. *J Am Acad Dermatol* 1989;20(2 Pt):257–60.

Issue with no volume

Baumeister AA. Origins and control of stereotyped movements. *Monogr Am Assoc Ment Defic* 1978;(3):353–84.

No issue or volume

Danoek K. Skiing in and through the history of medicine. *Nord Medicinhist Arsb* 1982:86–100.

Pagination in Roman numerals

Ronne Y. Ansvarsfall. Blodtransfusion till fel patient. *Vardfacket* 1989;13:XVI–XXVII.

Type of article indicated as needed

Spargo PM, Manners JM. DDAVP and open heart surgery [letter]. *Anaesthesia* 1989;44:363–4.

Fuhrman SA, Joiner KA. Binding of the third component of complement C3 by *Toxoplasma gondii* [abstract]. *Clin Res* 1987;35:475A.

BMJ Journals style guide and references

Article containing retraction

Shishido A. Retraction notice. Effect of platinum compounds on murine lymphocyte mitogenesis [Retraction of Alsabu EA, Ghalib ON, Salem MH. In: *Jpn J Med Sci Biol* 1979;32:53–65]. *Jpn J Med Sci Biol* 1980;33:235–7.

Article retracted

Alsabtr EA, Ghalib ON, Salem MH. Effect of platinum compounds on murine lymphocyte mitogenesis [Retracted by Shishido A. In: *Jpn J Med Sci Biol* 1980;33:235–7]. *Med Sci Biol* 1979;32:53–65.

15. Article containing comment

Piccoh A, Bossatti A. Early steroid therapy in IgA neuropathy: still an open question [comment]. *Nephron* 1988;48:289–91. Comment in: *Nephron* 1989;51:12–7.

Article commented on

Kobayashi Y, Fujii K, Hiki Y, *et al.* Steroid therapy in IgA nephropathy: a retrospective study in heavy proteinuric cases [see comments]. *Nephron* 1988;48:12–17. Comment in *Nephron* 1989;51:289–91.

Article with commentary

Information on commentaries can go in square brackets at end (but before date, if it's a reference in a letter):

Britto J, Nadel S, Maconochie I, *et al.* Morbidity and severity of illness during interhospital transfer: impact of a specialised paediatric retrieval team. *BMJ* 1995;331:836–9. [With commentary by S Logan] (30 September)

Article with published erratum

Schofield A. The CAGE questionnaire and psychological health [correction appears in *Br J Addict* 1989;84:701]. *Br J Addict* 1988;83:751–4.

Preprint articles

Preprints should always be indicated in the reference list by square brackets as follows:

Bar DZ, Atkash K, Tavarez U, Erdos MR, Gruenbaum Y, Collins FS. Biotinylation by antibody recognition. A novel method for proximity labeling. *BioRxiv* 069187 [Preprint]. 2016. <https://doi.org/10.1101/069187>

Books and other monographs

Personal author(s)

Colson JH, Armour WJ. *Sports injuries and their treatment*. 2nd rev edn. London: St Paul, 1986.

Editor(s), compiler as author

Diener HC, Wilkinson M, eds. *Drug-induced headache*. New York: Springer-Verlag, 1988.

Organisation as author and publisher

Virginia Law Foundation. *The medical and legal implications of AIDS*. Charlottesville: The Foundation, 1987.

Chapter in a book

Weinstein L, Swartz MN. Pathologic properties of invading micro organisms. In: Sodeman WA Jr, Sodeman WA, eds. *Pathologic physiology mechanisms of disease*. Philadelphia: Saunders, 1974: 457–72.

If an article has been republished in a book, cite the article, rather than the book.

BMJ Journals style guide and references

Conference proceedings

Vivian VL, ed. Child abuse and neglect: a medical community response. Proceedings of the First AMA National Conference on Child Abuse and Neglect; 30–31 March 1984, Chicago. Chicago: American Medical Association, 1985.

Conference paper

Harley NH. Comparing radon daughter dosimetric and risk models. In: Gammage RB, Kaye SV, eds. Indoor air and human health. Proceedings of the Seventh Life Sciences Symposium; 29–31 October 1984, Knoxville (TN) Chelsea (MI): Lewis, 1985:69–78.

Scientific and technical report

Akutsu T. Total heart replacement device. Bethesda (MD) National Institutes of Health, National Heart and Lung Institute; 1974 Apr, Report No: NIH-NHLI-69–2135–4.

White paper

Style for a white paper follows this model: Department of Health. Reforming the Mental Health Act. London: Stationery Office, 2000. (White paper)

Saying (White paper) isn't necessary if it is referred to as a white paper in the text. Please note, sometime near the end of 1996 HMSO changed its name to The Stationery Office; in references please use Stationery Office (not the Stationery Office).

Government publications

Department of Health and Social Security. Influenza. London: DHSS, 1979 (CMO (70) 11).

Hansard references

Milburn A. House of Commons official report (Hansard). 1999 Dec 2: col 450.

Cite the speaker for relatively straightforward things such as oral or written answers, eg, Clark K. Community care. House of . . .

Clinical Evidence

Katzka D, Delaney BC, Forman D, *et al*. Gastro-oesophageal reflux disease. In: Clinical evidence. Issue 4. London: BMJ Publishing Group, 2000:253–65.

Cochrane Collaboration Handbook

One of the publication types on the CD but doesn't get updated the way the systematic reviews are. Earliest version seems to be:

Sackett DL, ed. The Cochrane Collaboration handbook. Oxford: Cochrane Collaboration, 1994.

For later versions, use this format: Mulrow CD, Oxman AD, eds. Cochrane Collaboration handbook. In: Cochrane Library. Issue 4. Oxford: Update Software, 1997.

If the author isn't using the up-to-date version, query it.

Most Cochrane references are NOT cited as "In: Cochrane Library", etc – they are cited as Cochrane Database Syst Rev, etc.

Dissertation

Youssef NM. School adjustment of children with congenital heart disease [dissertation]. Pittsburgh (PA), University of Pittsburgh, 1988.

It should have the number of pages: 1998:1–280. The page numbers are useful for someone wanting to read it, or borrow it through interlibrary loan. Give the type of dissertation – Msc, PhD if the author does.

BMJ Journals style guide and references

Patent

Harred JF, Knight AR, McIntyre JS, inventors. Dow Chemical Company assignee. Epoxidation process. US patent 3,654,317. 4 April 1972.

Other published material

Newspaper article

Rensberger B, Specter B. CFCs may be destroyed by natural process. *The Washington Post* 7 August 1989; Sect A:2 (col 5).

Press releases

These can be cited in text (Middlesex Hospital, press release, 14 August 1990) or in reference list (especially when they have URLs or are part of a range of references)

Audiovisual

AIDS epidemic: the physician's role [video recording] Cleveland (OH): Academy of Medicine of Cleveland, 1987.

Computer file

Renal system [computer program] MS DOS Version. Edwardsville (MS) Medi-Sim, 1988.

Legal material

Toxic substances Control Act: Hearing on S776 Before the Subcommittee of the Environment of the Senate Comm. on Commerce, 94th Congress 1st September (1975).

Washington v Glucksberg 521 US 702 (1997)

Map

Scotland [topographic map]. Washington National Geographic Society, 1981.

Sacred text

Ruth 3:1–18 The Holy Bible. Authorised King James version. Oxford University Press, 1972.

Dictionary and similar references

Ectasia Dorland's illustrated medical dictionary 27th edn. Philadelphia: Saunders, 1988:527.

Classical material

The Winter's Tale: act 5, scene 1, lines 13–6. The complete works of Shakespeare, London: Rex, 1973.

Databases

Please reference as: Authors. Title of paper. Cochrane Library. Issue 3. Oxford: Update Software, 1997.

Example: McLellan J, Heneghan CJ, Perera R, *et al*. B-type natriuretic peptide-guided treatment for heart failure. *Cochrane Database Syst Rev* 2016;12:CD008966.doi:10.1002/14651858.CD008966.pub2.

In press/unpublished material

Lillywhite HB, Donald A. Pulmonary blood flow regulation in a snake. *Science*. In press.

British National Formulary (BNF).

Joint Formulary Committee. *British National Formulary*. Edition 75. London: BMJ Group and Pharmaceutical Press, March 2018.

BMJ Journals style guide and references

Summary of product characteristics

Panadol original tablets. Summary of Product Characteristics, UK. GlaxoSmithKline Consumer Healthcare (UK) Trading Limited, January 2018.

Source: <https://www.medicines.org.uk/emc>

Website/electronic material

Websites are referenced with their URL and access date. Access date is important, as websites can be updated and URLs change. The "data accessed" can be later than the acceptance date of the paper, and it can be given as the month the site was accessed.

When we give a web reference as well as a print one (eg, to a government document or to a journal reference) then we don't need to put date accessed (even if the web reference disappears the print reference remains).

Example:

Moore A. Paracetamol: widely used and largely ineffective [online]. 2018. Available:

<http://uk.cochrane.org/news/paracetamol-widely-used-and-largely-ineffective> [Accessed 23 May 2018].

Electronic journal articles: Morse SS. Factors in the emergence of infectious diseases. *Emerg Infect Dis* 1995 Jan-Mar;1(1). <http://www.cdc.gov/ncidod/EID/vol1no1/morse.htm> (accessed 5 Jun 1998).

Electronic journal; abstract only in printed version: *Pediatrics* 1999;103:e28.

This is the reference to the electronic version (in the printed version the abstract is on p 656; a librarian could find the reference from this information).

When electronic and print versions differ: Williams S, Poulton R. Twins and maternal smoking: ordeals for the fetal origins hypothesis. Abridged version: *BMJ* 1999;318:897–900. Full version: <http://www.bmj.com/cgi/content/full/318/7188/897>.

Rapid/electronic responses: Author. Title of rapid response [electronic response to First author *et al.* Title of the article]. *BMJ* year. URL (accessed 29 Feb 2000).

For a reference to all rapid/electronic responses to a particular article, the style is: Electronic responses. Title of article. *BMJ* year. URL (accessed 3 Mar 2000)

The accessed date is important as more responses might subsequently be added.

Department of Health References: Available online when there is a Gateway number: Department of Health. Best practice guidelines for doctors and other health professionals on the provision of advice and treatment to young people under 16 on contraception, sexual and reproductive health. 2004. <http://www.dh.gov.uk> (Search for: 3382).

General guidance

References cited only in tables or figure legends should be numbered in accordance with the sequence established by the first identification in the text of the particular table or illustration.

equally

References to television, radio in text: (Series, "Programme", BBC1, 2 Aug 92).

Software – treat as a drug: give manufacturer and place in parenthesis in text. Major types of software, such as SPSS, are OK without a reference.

Excel and Word - no need to raise an author query regarding version number.

CD Roms are publications and should be italicised, but don't use italics for online sources (eg, Medline).

Page numbers in references: when you need to repeat the same reference (to a book, usually) with different page numbers, consolidate these so that the source is referred to only once. It should then be referenced in the text as, for example: (Miller, p47)¹ and then (Miller, p89)¹

If referring to different chapters in a book with editors, give separate references to separate chapters.

When authors are study groups, they should be given in the authorial position, not left after the title as Medline does:

BMJ Journals style guide and references

- North American Symptomatic Carotid Endarterectomy Trial Collaborators. Beneficial effect of carotid endarterectomy in symptomatic patients with high-grade carotid stenosis. *N Engl J Med* 1991;325:445–53. [not: Beneficial effect of carotid endarterectomy in symptomatic patients with high-grade carotid stenosis. North American Symptomatic Carotid Endarterectomy Trial Collaborators. *N Engl J Med* . . .]
- European Carotid Surgery Trialists' Collaborative Group. MRC European Carotid Surgery Trial. Interim results for symptomatic patients with severe (70-99%) or with mild (0-29%) carotid stenosis. *Lancet* 1991;337:1235–43. [not: MRC European Carotid Surgery Trial. Interim results for symptomatic patients with severe (70-99%) or with mild (0-29%) carotid stenosis. European Carotid Surgery Trialists' Collaborative Group. *Lancet* . . .]

References to *J Bone Joint Surg* – authors often give them as, eg, *J Bone Joint Surg* 1991;73-B:287–90. You can't find these in PubMed unless the A or B is specified in the title – use *J Bone Joint Surg Am* and *J Bone Joint Surg Br*.